



Health & Social Services Department

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Adult and Child Services - Eligibility & Employment Services
Substance Abuse Services

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EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #6100

EFFECTIVE DATE: 8/01/93

APPROVED BY:

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REVIEWED BY:

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SUBJECT: INSTRUCTIONS FOR COMPLETION OF PATIENT CARE RECORD (PCR)

AUTHORITY: CALIFORNIA ADMINISTRATIVE CODE, TITLE 22, DIVISION 9, CHAPTER 2, ARTICLE 6, 100086; CHAPTER 4, ARTICLE 6, 10069.

PURPOSE/POLICY:

To establish written instructions for completion of the PCR.

**SOLANO COUNTY
EMERGENCY MEDICAL SERVICES
AGENCY**

**Patient Care Report (PCR) Form
Instruction Booklet**

Thomas L. Charron, M.D., M.P.H.

Solano County EMS Office
275 Beck Ave.
Fairfield, CA 94533
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August 1, 1993

Version 2.7

Attached is a copy of the revised Solano County Patient Care Report form. It was approved by the Solano county Health Officer/EMS Medical Director and is effective on or before August 1, 1993. The purpose of the form is to provide a standardized format to document Prehospital care, to provide standardized language for radio communication and to provide an instrument for the gathering of data for evaluation and EMS system productivity. Additionally, this form is to be used on all patient contacts including non-critical, pre-scheduled patient transfers and patient contacts which do not result in transport. (See Solano County Policy #6100). The following directions are a step-by-step procedure for filling out the form. Please read the instructions carefully and refer to the attached examples illustrating the proper use of the forms. These instructions are also designed to provide you with new information about documenting the patient encounter. Remember that even the most skilled and conscientious health care professional may have to go to court as a witness or even a defendant. In these circumstances the best protection is a thorough and accurate medical record. Be precise and as detailed as possible. A medical record may become a legal document that reflects upon its author. Any questions about the form or its use may be directed to the Solano County EMS Agency office at (707) 784-8155.

PATIENT INFORMATION SECTION

1. **Patient # of** refers to the number of patients at the scene of an incident. If the patient is a single patient at an incident, use: Patient 1 of 1. If there is more than one patient involved, use the appropriate count of patients on scene, e.g., patient 2 of 5, 1 of 3, etc.
2. **Agency** – Enter the name of the ambulance company. Each ambulance company has a code number assigned it by the Solano EMS office. You should enter either the actual company name or its EMS code number.
 1. Benicia Fire Dept.
 3. Foothill Ambulance (Sacramento Life Support)
 4. MEDI Ambulance Service
 5. Solano Ambulance Service
 6. Vacaville Fire Dept.
 7. Mare Island Fire Dept.
 8. Winters Fire Department
 9. Military Ambulance (Mare Island Branch Clinic or Travis AFB)
 20. Other Ground EMS Agency
 90. CALSTAR Air Ambulance\
 91. UCD Life Flight
 92. CHP EMS Aircraft (H-30)
 93. REACH Air Ambulance
 99. Other EMS Aircraft
3. **Unit #** - Enter the Solano Co. Dispatch Unit ID@ assigned to ground ambulance when the unit is permitted and becomes operational in the County. The Unit ID# should be used when coordinating an ambulance response with the County Communication Center and when the CMED becomes operational.
4. **Incident Number** – Centralized Medical Emergency Dispatch # (CMED#) when central dispatch goes on line. Refers to the number assigned by the dispatching agency to the incident which should be provided to the field crew by the dispatcher. At the present time, this number should be the unique incident number assigned by your ambulance company dispatcher.
5. **Date** – is indicated by month, day and year: e.g. 03/23/88.
6. **Time Onset** – refers to the known or estimated time of onset of major symptoms, or the time of the accident or injury. For cardiac arrest cases use this space to indicate the estimated time of onset of collapse. For trauma cases, time of onset will also be used to calculate time interval from incident until the EMS system arrives. This time can be actual or estimated. **Use (@) in front of the time if it**

is an estimate. Enter N/A for non-trauma incidents as appropriate (e.g., Chest pain, for 6 days). **This is not the time the ambulance service receives the call or is enroute to a call.**

7. **Duty** – (Data Point used for specific fire services in the County. Leave this box blank or enter N/A if applicable).
8. **849 Codes** (see following page for codes) – Refers to the ICD-9 coding system for coding of generalized Scene Descriptions of where an EMS case was found. The recommended codes for this data field are listed below and are on the attached Data Code Sheet available from the EMS office. Be as specific as possible.
9. **Payor Code** – Enter the insurance and/or expected payor codes of the patient. Enter UNK for unknown and N/A for non-applicable (e.g., dry runs, AMA, etc.) Enter either the actual pay source or the following standardized payor codes.

0	Unknown	7	Public Assistance
1	Kaiser	8	Self pay or uninsured
2	Other commercial insurance	9	Other
3	Medicare	10	Dual (Medi-Cal & Medicare)
4	State Medicaid (Medi-Cal)	20	Solano County Health Care Coalition
5	Champus	99	N/A
6	Workers Comp		

10. **Location** – refers to actual location of incident including either an address, landmark or intersection as well as specific location of victim. Please include city or town, if appropriate (e.g., 510 Main Street, Dixon – in back yard).
11. **Grid** – refers to the section of the County in which the incident is located. The Solano EMS System is currently using Thomas Brothers 1991 Digital Mapping System for grid locations for all requests for EMS Services. Enter the map page and grid coordinates of the location of the call in this box (e.g., Nut Tree Airport 433-F3).
12. **Reporting Party** – is the individual or Public Safety Answering Point (PSAP) that requested the ambulance to respond.

<u>PSAPs requesting EMS Response</u>		<u>Non-PSAPs Requesting EMS Response</u>	
0.	Unknown	10.	Private Request (non 911 request)
1.	PSAP Solano SO (911)	11.	Hospital Emergency Dept. Emergency Transfer
2.	PSAP Benicia (911)	12.	ECF/Nursing Facility (non 911 request)
3.	California Highway Patrol (911)	13.	Industry (non 911 request)
4.	PSAP Fairfield (911)	14.	TBA
5.	PSAP Suisun (911)	15.	CMF
6.	PSAP Vacaville (911)	20.	Kaiser Triage/Advice Staff
7.	PSAP Vallejo (911)	21.	Referral from another ground/air ambulance
8.	Other PSAP (Military, NAPA, etc).	22.	In Hospital Emergency Interfacility Transfer
		23.	In-Hospital Scheduled Interfacility Transfer
50.	Solano County CMED (when operational)	24.	Request from a Military Base for transfer
		99.	Other (non 911 request)

13. **Phone # of the Reporting Party** – as indicated (EXCLUDE PSAPs 1-8 & 50)
14. **Patient Name** – refers to the name of the patient: last, first and middle initial (i.e., Public, John Q.) Enter UNK if appropriate or N/A if the call resulted with a patient found at the scene of the emergency.
15. **Address, City, State, Zip Code and Phone** – Refers to the name of the patient's street and number, city or town, state, home zip code and telephone number. Enter UNK or N/A if appropriate. Home zip code should not be overlooked and is a critical data element to be included in the PCR report.
16. **Responsible Party** – refers to the name of the party responsible for the patient: last and first names and middle initial; i.e., Public, John Q. Enter (S/A) same as above if applicable.

17. **Address, City, State, Zip Code, and Phone** – refers to the name of the responsible party's street & number, city or town, state, home zip code and telephone number. Enter (S/A) same as above if applicable.

INITIAL ASSESSMENT SECTION

SCENE DESCRIPTIONS EMS Case (849 Codes)

0. Unknown

Home

- 10 Residence
- 11 Apartment
- 12 Boarding House
- 13 Farm House
- 14 House (residential)
- 15 Garage
- 16 PVT Swimming Pool
- 17 Yard of Home
- 19 Trailer Park

Mine & Quarry

- 20 Mine & Quarry
- 21 Gravel Pit
- 22 Sand Pit
- 23 Tunnel Under Construction

Farm

- 25 Agricultural Site
- 26 Farm Building
- 27 Land Under Cultivation

Industrial Place & Premises

- 30 Industrial Place/premises
- 31 Building under Construction
- 32 Factory
- 33 Industrial Yard
- 34 Loading Platform (Factory)
- 35 Plant (industrial)
- 36 Rail Yard
- 37 Shop (Place of Work)
- 38 Warehouse
- 39 Other

Place for Recreation/Sport

- 40 Place for Recreation/Sport
- 41 Amusement Park
- 42 Baseball/football Field
- 43 Gymnasium
- 44 Playground (including schools)
- 45 Public Park
- 46 Swimming Pool (public)
- 47 Vacation Resort
- 48 Campground
- 49 Other

Street

- 50 Street and/or Highway
- 51 Freeway/Highway
- 52 City Street
- 53 Rural Road
- 54 Non-Paved Road/Trail
- 55 Other

Public Building

- 60 Public Building
- 61 Airport
- 62 Café/restaurant
- 63 Church
- 64 Market/Shop
- 65 Office/Office Building
- 66 School
- 67 Hotel/Motel/Inn
- 68 Other

Medical Facility

- 71 Hospital
- 72 Nursing/Convalescent Home
- 73 Doctor's Office
- 74 Urgent Care Center
- 75 Outpatient Surgery Clinic
- 76 Other Type Clinic
- 79 Other

Residential Institution

- 80 Residential Institutions
- 81 Children's Home
- 82 Dormitory
- 83 Jail
- 84 Retirement Home
- 85 Prison (CMF)
- 86 Derelict House

Other Specified Places

- 90 Other Specified Places
- 91 Beach
- 92 Canal
- 93 Lake/reservoir
- 94 Parking lot/place
- 95 River
- 96 Military Base
- 99 Other

18. **Chief Complaint** – refers to the patient’s impression of the major symptoms responsible for the ambulance call. This is a record notation of what the patient tells you. A patient with an extremity injury may have a chief complaint of back pain, and if he has not noticed his extremity injury, this injury might not be a part of the chief complaint. Examples of chief complaints include: shortness of breath, pain right leg, headache, chest pain on inspiration, fainting, etc. The following would not be considered typical chief complaints: fractured femur, CVA, deformity of right wrist. If the patient is unconscious, indicate accordingly as the chief complaint for this category.
- Current History** – refers to your summation of the patient’s narrative of his/her medical problem, and any other information that pertains to the problem which is not a part of the physical findings. Include here such information as: what brought on the symptoms?; is the pain sharp or dull; does the pain radiate; did the patient get sweaty; as well as descriptions of the events surrounding the accident, secondary complaints, past medical history which relates to the present illness, major unrelated medical problems, recent use of medications, observations by bystanders, and details of the accident scene such as bent steering wheel. **Information which you get by observing or examining the patient does not belong under history; it belongs under physical findings or vital signs/treatment and response sections.**
19. **Prior Illness/Injury** – refers to the history as given by the patient, family or bystanders.
Past Medical History – refers to those conditions for which the patient is already under treatment.
20. **Age** – refers to patient’s age in years or months using mos. when months are indicated, e.g., y mos.
DOB – refers to date of birth by month, day and year.
Sex – refers to male or female, using M or F.
Weight – refers to patient’s estimated weight, using Kg when indicating kilograms.
21. **Mechanism of Injury Code** (see following page for codes) – Fill in this section only when trauma is involved. This includes falls, burns, electric shocks, etc. The codes will be found on the attached laminated code sheet provided by the EMS agency for each unit. It can be kept on a clipboard used to carry the Prehospital report forms. You may enter a brief statement regarding the mechanism of injury or the injury code attached (see following page). It is possible to have multiple mechanisms of injury for a single patient. Select the most significant as the prime mechanism of injury when such situations occur (e.g., a motor vehicle accident case who received a gunshot that caused the accident). Enter N/A if appropriate. Use UNK if mechanism is Unknown.
22. **Safety Device** – Fill in this section if you can document from history and/or inspection of the scene (e.g., seat belts, motorcycle helmets, protective eye wear for industrial injuries, protective clothing, air bags, etc).
0. Unknown
 1. None – inappropriate or non-use of safety device (e.g., MVA w/o seatbelt, MCA w/o helmet).
 2. Safety belt harness
 3. Air Bags
 4. Child Safety Seat
 5. Helmet
 6. Padding/Protective clothing
 7. Protective eye wear
 8. Other
 9. Not applicable (no appropriate device available; e.g., fall in bathroom).

MECHANISM OF INJURY CODES**100 Motor Vehicle Accidents**

- 101 MV vs. Train
- 102 MV vs. MV
- 103 MV vs. MC
- 104 MV vs. Animal Rider
- 105 MV vs. Pedal Cyclist
- 106 MV vs. Pedestrian
- 107 MV vs. Road Object
- 108 MV Occupant Ejection
- 109 NV vs. Other
- 110 MV rollover

120 Motorcycle Accidents

- 121 MC vs. MV
- 122 MC vs. Pedestrian
- 123 MC vs. Highway Object
- 124 MC vs. Other
- 125 MC Occupant Ejection

130 Other Transportation Acc.

- 131 Water, Craft vs. Craft
- 132 Water, Craft vs. Water Skier
- 133 Water, Craft vs. Swimmer
- 134 Water, Craft Fall/Submersion
- 135 Water, Craft, Noncoll/ Fire/Explosion
- 136 Water, Craft (other)
- 140 Air, Powered Craft
- 142 Air, Unpowered Craft accident
- 143 Air, Parachutist, Accident
- 144 Pedal Cycle Accident
- 145 Ridden Animal Accident
- 146 Train or Rail Accident
- 147 Bus
- 148 Other Powered Vehicle
- 149 Other Vehicle not elsewhere classified

200 Accidental Fall**220 Injury Caused by fire or flames****240 Accidents by Submersion/ Suffocation/Foreign Bodies**

- 241 Acc Drowning & submersion
- 242 Acc Foreign Body, eye
- 243 Acc Foreign Body, airway

250 Other Accidents

- 251 Struck, falling object
- 252 Struck, Object/Person, sports
- 253 Caught between moving/stationary objects
- 254 Machine Injury
- 255 Cut/Pierce, powered lawn mower
- 256 Cut/Pierce, other powered tool
- 257 Cut/Pierce, Knife/Sword/Dagger
- 258 Acc. Cut/Pierce, other object
- 259 Acc. Explosion, Boiler
- 260 Acc. Explosion, Gas Cylinders
- 261 Acc. Explosion, Pressure Vessel, Other Spec.
- 262 Acc, Firearm, Missile, Other
- 263 Acc. Explosion, Fireworks
- 264 Acc. Explosion Blasting Material
- 265 Acc. Explosion, Gases
- 266 Acc. Explosion, Other Material
- 267 Acc. Burn, Hot Liquid/Vapor

- 268 Acc. Burn, Caustic/Corrosive
- 269 Acc. Burn, Other
- 270 Acc. Elec Current, Other
- 271 Acc. Radiation Injury
- 272 Movement, Overexertion/Strain
- 273 Environmental, other

300 Suicide/Self-Inflicted Injury

- 301 Suicide, Hanging
- 302 Suicide, Firearm
- 303 Suicide, Cut/Pierce Instrument
- 304 Suicide, Jump
- 305 Suicide, other

320 Injury Inflicted by Other Person

- 321 Assault, Unarmed flight/brawl
- 322 Assault, Rape
- 323 Assault, Corrosive/Caustic
- 324 Assault, Firearm
- 325 Assault, Cut/Pierce Instrument
- 326 Struck, Blunt/Thrown Obj
- 327 Criminal Neglect/Child Abuse
- 328 Assault, Other
- 340 Legal Intervention

999 Other Mechanism of Injury Not Listed

N/A Not applicable (no external Trauma and/or mechanism of Injury – medical emergency.

23. **First Responder and Treatment Before Arrival** – refers to any treatment or medication given prior to arrival of responding unit and given by whom, including first responders (e.g., CPR in progress by Acme Fire Department; Nitro 1 TAB P.O. given by husband; extrication by CHP). It is becoming more important to identify the actual organization providing medical care before the arrival of the first responding ambulance unit. Listed below are standardized code numbers of various EMS service providers that should be included in the medical record of EMS care was provided before your arrival. A brief statement that summarizes pre-arrival care is all that is needed. In the near future care provided by many fire first responders shall be included in the records of the other agencies that responded to the call. **First Responder Codes are listed below.**

- | | | |
|--|----------------------------------|--|
| 0 Unknown (first responder & level of care are not known) | 13. Fairfield City Fire Dept | 31. CHP |
| 1. Benicia Fire Dept ALS | 14. Suisun City Fire Dept | 32. Solano S.O. |
| 3. Foothill Ambulance (Sacramento Life Support) | 15. Cordelia Volunteer Fire Dept | 40. Industrial Safety or Fixed First Aid Station |
| 4. Medic Ambulance | 16. Dixon City Fire Dept | 41. Marine World Africa USA |
| 5. Solano Ambulance | 17. Winters Fire Dept | 50. Citizen, General Public and/or Family Member |
| 6. Vacaville Fire Dept ALS | 18.. Rio Vista Fire Dept. | 51. Citizen who received pre-arrival instructions from PSAP Dispatcher |
| 8. Other Ground Ambulance Service | 19. Vacaville Rural Fire Dist. | 60., Individual who was injured or ill. |
| 9. Other Air EMS Aircraft Service | 20. Mare Island Fire Dept | 90. Medical staff at a medical facility where patient was found |
| 10. Fire Dept First Responder (Generic or unable to determine) | 21. Suisun Rural Fire Dept | 98. Other (NOS) |
| 11. Vallejo Fire Dept. | 22. CMF Fire Dept | 99. Clearly NO first responder was present |
| 12. American Canyon Fire Dept | 23. CDF Fire Dept | |
| | 24. EXXON Fire Dept. | |
| | 25. Montezuma Fire Dept. | |
| | 30. Law Enforcement | |

24. **Medications** – refers to any prescription medication taken regularly by patient. If none, state so.
Allergies – refers to any substance the patient has as a known allergy. If none, state so.

25. **Hx of Substance Abuse or Hx of ETOH** – If determined to be positive from a secondary assessment, obtained from patient history, or verified from observing the scene, you are required to include a notation of your professional finding if you suspect that there is a history of substance abuse and/or alcohol ingestion associated with the call. This assessment is not a definitive diagnostic assessment nor is it a legal interpretation that the patient is intoxicated. This is your assessment of the clinical

status of the patient and a Prehospital medical finding that can and will have a significant impact on both your Prehospital care and any clinical decision regarding patient refusal of EMS services. Your patient medical record is not a public document and you are required to include all Prehospital assessments and all care on the medical record. The information required from these two data elements are being used to monitor the impact alcohol and substance abuse has upon the EMS system. Refusal to include this information on the medical record is viewed as criminal violation of Section 471.5 (Falsification of medical records). EMT-Ps or EMT-1s refusing or electing not to provide this critical information on their medical records are subject to both formal disciplinary action as allowed by 1798.200 (e) and (g) of the Health and Safety Code and possible referral to the district attorney for criminal prosecution.

26. **Position Found** – Enter a stick figure or a general statement regarding the physical placement of the patient when you first arrive on scene (e.g., standing, sitting or prone).
27. **PMD** – (Private Medical Doctor) enter name as appropriate.
28. **Injury Codes** – Enter the injury codes on the body/baby graphics as appropriate (see PCR).
29. **Explanation of Level of Consciousness (GCS)/Physical Findings** – refers to any additional information to further explain the injury code drawings on the body graphics and Glasgow Coma Score.
30. **Initial Coma Score** – refers to Glasgow Coma Scoring; circle the appropriate box as determined by your assessment.

VITAL SIGNS/TREATMENT AND RESPONSE

31. **Degree of Severity** – Using definitions listed below, classify your impression of the patient's initial severity upon arrival:
 - **Immediate – requires immediate medical intervention(s)** – Any clinical case where an EMT-P or EMT-1 determines a patient needs immediate medical (Advanced or Basic Life Support) intervention while performing a primary survey (airway obstruction, respiratory distress requiring ventilation, chest pain with significant life threatening arrhythmias, etc.).
 - All cases with significant airway, respiratory and cardiac compromise (assessed during a secondary survey) in which the time needed to obtain a Base consult will result in further clinical compromise and/or biological death.
 - All cases assessed as critically ill or injured (or immediate as defined by S.T.A.R.T. criteria) during MCIs.
 - All cases where resuscitation is initiated immediately and later discontinued (by Base Hospital order or presentation of valid DNAR documentation).
 - **Delayed – may require ALS definitive medical intervention** – but the patient's condition is not expected to clinically change until after Base Hospital consult has occurred or the patient is transported to a receiving facility.

These patients may have unstable or questionable vital signs but do not present with an obvious degree of distress and/or need for pain relief that would benefit from immediate pre-hospital (ALS) EMS intervention.
 - **Stable (minor or moderate illness/injury)** – requires transport and comfort measures only. These cases are clinically assessed as non-emergent cases requiring EMS assistance but not immediate ALS intervention. There may be a need to initiate ALS care for these cases, but Base consult is indicated because the need is only for patient comfort and relief. There is usually no concern for loss of life or limb associated with EMS care rendered. Immediate ALS intervention is not commonly indicated without Base consultation except for extraordinary circumstances.

- **DOA (non-resuscitation case)** – these cases are classified under EMS Policy #6140, Determination of Death.
 - **Unable to Classify** – EMT-P unable to determine condition; **should be an exceptional case.**
32. **Airway, breathing, lung sounds, pupil response, skin color, skin temp, skin moisture, head, neck, ears/nose, thorax, abdomen, pelvis & extremities** – Check boxes as appropriate, write in clinical finding(s) as appropriate.
33. **ET Intubations** – enter data as appropriate if intubation was attempted on this patient.
34. **IV Lines** – enter data as appropriate if IV lines were attempted on this patient. If unable to initiate and/or the Base Hospital requests an IV to be started, but events in the field prohibit the EMT-P from starting the line, place UTE (Unable to Establish) in the volume infused box.
35. **Communications** – Identify the type of equipment used for medical communications with the Base Hospital, as well as the time of Base Hospital contact for ALS calls. BLS calls should also include the time the receiving hospital was notified when appropriate.
36. **Vital Signs/Orders/Treatment/Response Section** – Use this space to record care givers' Solano County Authorization #, time, procedure/medication/pt. response/EKG rhythm strip interpretation, and vital signs. Record all subsequent vital signs or rhythm strips (with times) in this section. Include all other Base orders and patient responses to treatment in this section.
37. **Open Space** – to be used for any additional written comments as appropriate.
38. **Continuation Form** – check this box if a continuation form is utilized for this EMS call.

DISPOSITION SECTION

39. **Clinical Impression** – Enter your over-all clinical impression of the patient's clinical condition. This is a critical data element needed for retrospective medical audit.
40. **Treatment Code(s)** (see following page for codes)– List the ALS or BLS clinical protocols (treatment guidelines) utilized as standards of care this patient received. The codes for these conditions are listed on the laminated sheet provided to each unit (e.g., Bradycardia = C6). See the following page for a listing of ALS treatment protocols used. BLS treatment protocols have the same code except use a lower case letter for the code.
41. **BLS/ALS** – refers to emergency calls with a level of care provided to the patient in accordance with CA Health and Safety Code 1797.52 (ALS) and 1797.60 (BLS). Limited advanced life support is to be considered ALS. (Calls in which EMT-Ps are performing paramedic assessments but providing BLS care are considered BLS calls).
42. **Responding From Station Grid** – check the box when the unit dispatched has departed from its quarters/station and/or a preassigned posting/staging position using the County's ambulance system status management plan. The code numbers of each posting position shall be made available when the CMED is operational. Include the station identification and/or prearranged position identification code in this box.

ALS TREATMENT PROTOCOLS**Cardiac Emergencies**

- C1 Shock (Non-Traumatic)
- C2 Shock (Non-Trauma & Resp Dist)
- C3 Cardiac Arrest (V-fib/V-tach)
- C4 Cardiac Arrest (PIVR, EMD, Asystole)
- C5 Vent Tachy with Pulses
- C6 Bradycardia with Pulses
- C8 Paroxysmal Supra-vent Tach
- C9 Other Cardia Cysrhythmia
- C10 Chest Pain (Cardiac)
- C11 Chest Pain (Non-Cardiac)
- C12 Hypertensive Emergency

Environmental Emergencies

- E1 Heat Illness/Hyperthermia
- E2 Hypothermia
- E3 Envenomation
- E4 Burns
- E5 Near Drowning/Drowning

**HAZ-MAT Emergencies
(See Special H Codes)**

- H1 General Procedures
- H2 Irritant Gases
- H3 Hydrofluoric Acid
- H4 Smoke Inhalation/Carbon Monoxide
- H5 Arsine/Phosgene Gas
- H6 Cyanide
- H7 Hydrogen Sulfides & Mercaptans
- H8 Petroleum Distillates
- H9 Pesticides

Medical/Neurological Emergencies

- M1 Abdominal Pain
- M4 Allergic Reaction
- M5 Anaphylaxis
- M6 Poisoning/Overdose
- N1 Coma/ALOC
- N2 Diabetic Complications
- N3 Seizures
- N4 CVA/Stroke
- N5 Syncope

Obstetric Emergencies

- O1 Vaginal Hemorrhage with Shock
- O2 Vaginal Hem w/o Shock
- O3 Imminent Delivery, N Present
- O4 Breech Present/Prolapsed Cord
- O5 Pre-eclampsia/Eclampsia

Pediatric Emergencies

- P1 Ped Cardiac Arrest (V-fib/Tach)
- P2 Ped Cardiac Arrest (EMD/AS)
- P3 Pediatric Hypotension
- P4 Neonatal Resuscitation
- P5 Pediatric Bradycardia

Behavioral Emergencies

- B1 Psychiatric Crisis
- B2 Child Abuse/Sexual Assault

Respiratory Emergencies

- R1 Airway Obstruction
- R2 Croup/Epiglottitis
- R3 Acute Respiratory Distress
- R4 Respiratory Arrest
- R5 COPD
- R6 Asthma/Bronchospasm
- R7 Acute Pulmonary Edema
- R8 Spontaneous Pneumothorax
- R9 Toxic Gas Inhalation

Trauma Emergencies

- T1 Traumatic Shock
- T2 Traumatic Cardiac Arrest
- T3 Head and Neck Trauma
- T4 Chest Trauma
- T5 Abdominal Trauma
- T6 Extremity Trauma

Special Procedures

- S1 Plueral Decompression
- S2 High Dose Epinephrine
- S3 Rectal Diazepam (Valium)
- S4 Needle Cricothyrotomy
- S5 Adensoine (TBA)
- S6 Interosseous Line (TBA)

43. **Dispatch Priority** – Policy 5520 requires dispatchers of EMS units to assign a call priority classification number for each emergency call. This priority number should be given when the call is dispatched. Ambulance units enroute to a call by not on-scene may be redirected to a higher priority if clinically appropriate or directed by the CMED. Enclosed is a listing of the standard dispatch priority numbers:

<u>Priority Level</u>	<u>EMD assessment of patient severity</u>	<u>Recommended Code Out</u>
Priority 1	A presumed life threatening emergency from call prioritization	Code 3
Priority 2	An emergency requiring immediate response but assessed not to be life threatening	Code 3
Priority 3	An EMS call for a presumed non-life threatening, but emergency condition	Code 2
Priority 4	An EMS call for a presumed life threatening emergency at a medical facility	Code 3
Priority 5	An EMS call, non-emergency and non-scheduled	Code 2
Priority 6	Scheduled EMS transport	Code 2
Priority 7	Special scheduled event standby	Code 2
Priority 8	Special ambulance transport with medical team being transported	Code 2 or 3

44. **Code Out** – refers to code enroute to scene, **Code In** – refers to code enroute to final destination. Check the appropriate boxes. Code 1 is used for pre-scheduled transfers. For calls that are upgraded from Code 2 to Code 3, check both the Code 2 and Code 3 boxes. For calls that are downgraded from Code 3 to Code 2, do likewise.
- Code 1 Pre-scheduled Interfacility transfer
 - Code 2 Response immediately but without lights and/or siren; these calls include non-scheduled transfers
 - Code 3 Immediate response with lights and siren
45. **MICN/MD** – as indicated.
BASE – as indicated (NBMC, SSMC, METH, SDH) Enter N/A if appropriate.
46. **Receiving Facility** – as indicated. Enter N/A if appropriate.
Received By – name or identification of the individual who received the patient from the EMS team.
47. **Received** – refers to time call is received by ambulance service (i.e., dispatch time)
Enroute – refers to the time field actually rolls the unit (wheels of the ambulance are moving).
On Scene – refers to the time field unit stops at the scene (wheels of the ambulance stop).
Depart Scene – refers to time field unit leaves the scene of the incident.
At Hospital – refers to time field unit stops at final destination with patient transported. (NOTE that if the call resulted in a dry run or EMS care was handed off to another transporting unit, e.g., ground or air ambulance, enter the time your patient care responsibility ended on this line).
Leave Hospital – as indicated.
Available – refers to the time when the field crew is available to take a new call.
48. **Final Disposition Code** – Enter code as appropriate for the final disposition of the call.

0 **Unknown**

EMS Unit Transport Codes

1. Transport by Unit to Emergency Dept.
2. Transport by Unit to Emergency Dept. (Diversion)
3. Transport by Unit to Hospital (non-ed)
4. Transport by Unit to Non-Hospital Medical Facility
5. Transport to a Nursing Home/ECF etc.
6. Transport by Unit to Other Location (e.g., residence)
7. Transport to Rendezvous Point for Air Ambulance Transport
8. Transport to Rendezvous Point for Ground Ambulance Transport
9. DNR patient transport w/o resuscitation

Patient Transport via Other Mechanism

10. Patient Transported by EMS another unit (ground)
11. Patient transported by EMS another unit (air)
12. Patient transported via law enforcement
13. Patient transported via non-EMS system (family, friend, etc).

No Transport/Call Terminated

20. Treatment/transport refused
21. Aid not necessary
22. Dead on Scene (DOA)
23. DNR case no transport
24. Dispatch terminated call
25. Standby (no patient)
26. Left at scene
27. Other
28. Special Case – Field resuscitation terminated by base hospital order (patient not transported)
29. Special Case – Field resuscitation terminated by base hospital after patient was enroute to a hospital.
98. N/A
99. Not Recorded

49. **Destination Decision Code** – Enter code as appropriate.

- 0 Unknown
1. Nearest Receiving Facility
2. Patient/Family Request
3. PMD Request.
4. Triage to Regional Center (Major Trauma, Burns, Spinal Injury, etc).
5. No Preference (Kaiser/Sutter-Solano Rotation)
6. Bypass/ED Closure
7. Base Hospital Directive
8. Other
9. N/A

50. **Special/Unusual Scene Conditions** – Check any and all of the appropriate boxes relating to the circumstances at the scene. Any refusal of care by patient must be documented and explained on this form. Transfer refers to prearranged or emergency Interfacility transfers where a patient is transported to a facility with a higher level of care. If the case is a transfer check either the ALS or BLS box and identify the level of care provided. Transports from doctor's offices or nursing homes to emergency departments when a transfer form (and transfer orders) are not used, are to be considered emergency calls rather than transfers. (Special codes will be provided by the EMS Agency).

10. ALS prior to Base Hospital Contact
11. Radio Communication Failure with On-Line Medical Control
12. Obesity requiring delay before initiating transport or patient movement
13. Language problem (delay on scene because cannot interpret)
14. Cognitive deficit (delay on scene because cannot effectively assess patient's medical needs).
15. Domestic violence case requiring EMS response.
16. Gang violence case requiring EMS response.
17. Pregnant patient needing EMS services
18. Continuation form
19. HAZ-MAT Incident
20. Unsafe Scene requiring delay before accessing the patient
21. Extrication was required delaying on-scene time
22. Donor card search
23. Donor card found
40. MD was present and on scene with patient
41. Special QI monitoring codes for Special Studies @ individual EMS services
42. **EMT-P administration of Morphine.**
59. End of Special QI Monitoring codes for special studies of EMS services.
60. An alternate Base Hospital was used.
61. EMS aircraft was used
- 62. Major Trauma patient**
63. Interfacility Transfer
70. MCI or Medical Disaster Incident
80. DNAR (Do Not Resuscitate)
90. 5150 Call
91. Special QA Monitoring Codes for EMS Agency and/or Base Hospital studies.
99. End of Special QA Monitoring Codes for EMS Agency and/or base Hospital special study.

- **SPECIAL NOTE:** The new PCR has a new check box for the EMT-P to identify trauma patients who the EMT-P evaluates may need the immediate services of a trauma center. The Solano County EMS System is currently in a planning phase attempting to develop a formal trauma system. Patients that EMT-Ps and EMT-1s assess as a major trauma patient needing specialized trauma services are currently being monitored by the Physician Forum to help plan and develop formal trauma triage criteria for the proposed trauma system. At that time, patients who meet Solano county Trauma Triage Criteria should have the box checked.

51. **Change in Patient Condition Upon Hospital Arrival** – Check the one most appropriate box. Use the “field save” box only for Prehospital cardiac or respiratory arrest which is successfully resuscitated to a perfusing rhythm with reasonable oxygenation. Definitions for each of the following classifications are listed below:

- **Field Save** – should be limited to cases assessed as immediate on the initial evaluation of a patient and the patient should be in a physiological state that clinical death is expected if EMS intervention could not be provided. The patient must have a pre-existing problem such as an airway obstruction, ventilation failure and/or cardiovascular insufficiency that is incompatible with life. The EMS intervention of these cases must result with the patient no longer at risk of clinical death and the patient's airway, ventilation and cardiovascular function(s) have successfully returned with the need for minimal secondary support from the EMTs caring for the patient. Examples of such cases include:
 - Successful removal of a foreign body from an obstructed airway;
 - Successful restoration of ventilation ability for an apneic patient
 - Successful defibrillation resulting with a functional cardiovascular output.
 - **Improved** – Should be limited to cases where patient's physiological and clinical condition has improved as a result of EMS intervention. The improvement should be measurable or shown to have a cause-effect relationship that can be easily determined by reviewing the PCR. For example:
 - A positive change in GCS or neurological function after initiation of treatment (e.g., Narcan, D₅₀)
 - A positive change in vital signs after the initiation of treatment (respirations, pulse and/or GCS).
 - A noticeable reduction in patient psychological distress/pain as noted on the PCR response area;
 - A positive change in cardiac rhythm (either elimination of ventricular ectopy and/or improvement in cardiac rhythm).
 - **No change** – self explanatory.
 - **Worsen** – Should be limited to cases where patient's physiological and clinical condition has deteriorated even with aggressive EMS intervention. The negative change should be measurable and easily determined by reviewing the PCR. For example:
 - A negative change in GCS or neurological function as a function of time.
 - A negative change in vital signs after initiation of treatment (respirations, pulse, and/or blood pressure, GCS).
 - A negative change in dysrhythmia (either increased ventricular ectopy and/or increased cardiac dysrhythmia).
 - **Unable to Determine** – Self explanatory.
52. **ID Letter, ID#** - The primary technician authorization # should be the actual ID# of the individual who was directly responsible for the level of care the patient received. **Field Preceptors sign as primary EMT because they are responsible for the level of care rendered by students and interns. Interns and students providing care should sign as the secondary or third technician as appropriate utilizing their individual authorization #'s. Any EMT-P who assists by providing definitive ALS care must also review and initial his/her authorization # on the PCR prior to submission to the receiving physician.**
53. **Patient Identification Addressograph** – this area is for the receiving facility to run the PCR over an addressograph plate for record identification.

BASE HOSPITAL COPY ONLY – The third layer of the patient care report form has a quality improvement monitoring component that is to be completed and submitted to the base hospital when clinically appropriate. Cases which require the initiation of advanced life support care without base consult and cases that resulted in field intubation are being closely monitored by the EMS system. This section of the PCR is not part of the patient's medical record and is limited to only one copy for Base hospital retrospective medical review. This section should not be photocopied, reproduced or distributed beyond the Base Hospital which is responsible for retrospective medical review of the ALS call. See the following page for an example of the Base Hospital PCR copy.

54. **ALS Prior to Base Contact** – refers to the information required to justify an ALS procedure prior to medical direction from the Base Hospital (Section 100144(e) CAC, written documentation shall be filed within 24 hours. List protocols by codes provided under section #37 of the PCR instructions. List all drugs, procedures and treatments performed before Base Contact.
55. **Retrospective Base Hosp Eval of ALS Prior to Base Contact** – to be filled out by Base Liaison after ALS call has been audited.
56. **Intubation Attempt Report** – this information to be filled out on all intubation attempts. Fill in and check all that apply.
57. **Verification of Intubation** – this information to be filled out by MD receiving the intubated patient.

OTHER PCR INFORMATION NOT SHOWN

(58) RELEASE OF MEDICAL RESPONSIBILITY

I THE UNDERSIGNED DO HEREBY KNOWINGLY AND WILLINGLY REFUSE;

- BASIC LIFE SUPPORT TREATMENT
- ADVANCED LIFE SUPPORT PROCEDURES
- TRANSPORT TO ANY MEDICAL FACILITY

HAVING BEEN ADVISED THAT THE ABOVE TREATMENT/TRANSPORT IS MEDICALLY NECESSARY AND MY REFUSAL MAY RESULT IN INJURY OR DEATH, NEVERTHELESS, I DO ABSOLVE SOLANO COUNTY E.M.S. SYSTEM

AMBULANCE SERVICE/FIRE DEPT. AND _____ BASE HOSPITAL PHYSICIAN/MICN OF ANY FURTHER RESPONSIBILITY.

SIGNATURE _____

REFUSED TO SIGN _____ DATE _____

WITNESS: _____

WITNESS: _____

NO COPY

(59) PHYSICIAN ON SCENE

After identifying yourself by name as a physician licensed in the State of California, and if requested, showing proof of identity, you may choose to do one of the following:

1. Offer your assistance with another pair of eyes, hands, or suggestions, but let the life support team remain under base hospital control; or,
2. Request to talk to the base station physician and directly offer your medical advice and assistance; or
3. Take total responsibility for the care given by the life support team and physically accompany the patient until the patient arrives at a hospital and responsibility is assumed by the receiving physician. In addition, you must sign for all instructions given in accordance with local policy and procedure (whenever possible, remain in contact with the Base Station Physician).

I, _____, A LICENSED CALIFORNIA PHYSICIAN _____ I HEREBY ACCEPT COMPLETE RESPONSIBILITY FOR

PATIENT: _____

FROM: _____
AMBULANCE SERVICE, FIRE DEPT AND _____ BASE HOSPITAL PHYSICIAN, MICN, ASSUMTE FULL RESPONSIBILITING INCLUDING PHYSICALLY ACCOMPANYING THE PATIENT DURING TRANSPORT AND SIGNING FOR ALL TREATMENT ORDERED GIVEN BY ME.

SIGNATURE: _____

1 COPY BASE

58. **Release of Medical Responsibility** – refers to that section found on the back of the original (provider) copy where AMA (against medical advice) documentation takes place. Please refer to Solano County EMS Policy #6120 for further instructions.
59. **Physician on Scene** – refers to that section found on the back of the original (provider) copy where information is documented by the Physician who participates in the patient care within the Prehospital setting.

**Solano County Patient Care Report (Base Hospital Q1)
EMT-P Advanced Life Support Report Forms**

(54)

Advanced Life Support prior to Base Hospital Consult Report Forms (EMT-P Scope of Practice 100144(d) & (d))

ALS Protocol Initiated Prior to Base Consult

Specific Advanced Life Support Treatments
Rendered Prior to Base Consult

Reasons for initiation of ALS treatment prior to a Base Hospital Consult (check all that apply)

- ☐ Patient's clinical status demanded immediate intervention prior to voice contact with a Base Hospital.
☐ Field communication equipment was not available at the patient's side.
☐ No immediate response from the Base.
☐ Patient's physical environment was not suitable for radio and/or landline medical communications.
☐ EMS communications equipment malfunction.
☐ Radio frequency interference
☐ Radio over-ride by another unit.
☐ Concurrent MCI and/or medical disaster communication needs to precedence over this case.
☐ Other (Please attach additional sheets as necessary):

Change in patient clinical status after Advanced Life Support initiated (check one):

- ☐ Field Save ☐ No measurable change/unchanged
☐ Improved ☐ Deteriorated

Submitted by _____ Auth # _____ Date: _____

(55)

Retrospective Base Hospital Evaluation – ALS Care administered by the EMT-P prior to Base Contact was:

- ☐ Acceptable, medically appropriate and positively impacted patient's clinical condition.
☐ Acceptable but EMT-P care contributed little or nothing to the patient's clinical situation;
☐ Not Acceptable (the independent decision of the EMT-P to implement ALS care prior to Base Consult had a negative impact upon the patient's overall medical condition. Care requires base liaison intervention/counseling).

Auditor _____

(56) Intubation Attempt Report: this section must be completed on all intubation attemptsAuthorization # of EMT-P(s) attempting intubation _____ Endotracheal Tube Size _____ # of attempts _____
Endotracheal intubation was successful? ☐ Yes ☐ No If YES, was an End-Tidal CO₂ Monitor used? ☐ Yes ☐ No.

Base Station Contact associated with the intubation

- ☐ Prior to first attempt
☐ After first attempt
☐ No contact with Base

Indication for Endotracheal Intubation

- ☐ Cardiac arrest ☐ Traumatic arrest
☐ Respiratory arrest ☐ Drowning
☐ Respiratory distress ☐ Other _____

(57) Verification of Intubation: Receiving Physician ONLYTube was correctly placed? ☐ YES ☐ NO Tube was appropriately anchored? ☐ YES ☐ NO Tube size was appropriate? ☐ YES ☐ NO.

Method of verification: _____

Complications (if any): _____

COMMENTS: _____

Receiving Physician Signature: _____ Date: _____

DEPARTMENT OF HEALTH & SOCIAL SERVICES

Medical Services Division

GERALD HUBER
Director

BRYN MUMMA, MD, MAS
EMS Agency Medical Director

EMERGENCY SERVICES BUREAU
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Suite 2400, MS 20-240
Vallejo, CA 94590



**SOLANO
COUNTY**

TED SELBY
EMS Agency Administrator

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www.solanocounty.com

POLICY MEMORANDUM 6105

Implementation Date: December 15, 2011

Revised Date: January 4, 2021

Review Date: January 4, 2023

REVIEWED/APPROVED BY:

A handwritten signature in blue ink, appearing to read "Bryn Mumma".

MD, MAS

BRYN MUMMA, MD, MAS, EMS AGENCY MEDICAL DIRECTOR

A handwritten signature in blue ink, appearing to read "Ted Selby".

TED SELBY, EMS AGENCY ADMINISTRATOR

SUBJECT: SOLANO COUNTY PREHOSPITAL TRAUMA TRIAGE PLAN

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.220, 1798.100, 1798.162, and 1798.163

PURPOSE:

To maintain a system that allows trauma patients to benefit from receiving Trauma Center services most appropriate to that patient's injuries in the most expeditious manner possible. Level I, II, or III Trauma Centers should be utilized when appropriate.

I. GENERAL CONCEPTS:

- A. Trauma Centers improve the outcomes for patients with significant traumatic injuries.
- B. Level I/II Trauma Centers are able to provide emergent neurosurgical intervention and have in-house trauma surgeons and operating room teams immediately available; therefore major trauma patients with injuries (definite or suspected) requiring immediate access to a neurosurgeon should be transported to the closest Designated Level I/II Trauma Center.

II. PATIENTS LIKELY TO BENEFIT FROM TRAUMA CENTER SERVICES:

A. Trauma patients to be transported to closest Level I or II Trauma Center:

1. Glasgow Coma Scale (GCS) ≤ 8 ; or
2. Penetrating trauma to head; or
3. Suspected open or depressed skull fracture; or
4. Paralysis.

B. Physiological Criteria to be transported to closest Designated Trauma Center:

1. Systolic blood pressure less than 100mm/Hg; or
2. Inappropriate pediatric vital signs; or
3. Not following commands – GCS 9 – 13; or
4. Pregnant patients ≥ 24 weeks with torso trauma.

C. Anatomical Criteria to be transported to the closest Designated Trauma Center:

1. Penetrating injury to neck, torso, buttock, groin, or extremities proximal to knee or elbow; or
2. Two or more proximal long bone fractures; or
3. Amputation/crush/degloving proximal to wrist or ankle; or
4. Pelvic instability; or
5. Pulseless extremity; or
6. Trauma with burns with TBSA $> 9\%$.

D. Mechanical Criteria to be transported to the closest Designated Trauma Center:

1. Falls greater than 20 feet for adults, or two times the height of a child; or
2. Ejection from a vehicle, partial or complete, with injury; or
3. Motor vehicle crash (MVC) with death in the same vehicle; or
4. Thrown from an animal with injury; or
5. Vehicle vs pedestrian/bicyclist thrown with impact at > 10 mph or torso run over; or
6. Motorcycle > 20 mph with impact to an object (excluding road).

- E. Transport to a trauma center should be considered in the following situations:
 - 1. Fall in patient ≥ 65 years old;
 - 2. Anticoagulant use.

III. TRAUMA MEDICAL DIRECTION

- A. Major trauma patients in the pediatric age range (less than 15 years of age) should bypass local Trauma Centers and be transported to a Pediatric Trauma Center unless they meet the criteria of Section II(B), (D) or V of this policy.
- B. Paramedics have been trained to apply protocols and use judgment to identify Level I/II trauma patients. The approved trauma triage algorithm will be used to determine the appropriate trauma center destination.
- C. When assessing, treating, or transporting a Level I/II trauma patient, Paramedics shall contact the Solano Emergency Medical Services Cooperative (SEMSC) designated Level II Trauma Base Hospital for medical direction if either SEMSC's protocols require securing medical direction or if Paramedics otherwise determine medical direction is necessary.
 - 1. The SEMSC designated Level II Trauma Center is Kaiser Foundation Hospital, Vacaville.
 - 2. For trauma patients originating in the Vallejo/Benicia area, factoring in time of day, traffic, weather, etc., trauma patients may be transported directly to John Muir Medical Center (JMMC), Walnut Creek. If a Paramedic is requiring medical direction and is transporting to JMMC, the Paramedic shall contact the SEMSC designated Level II Trauma Base Hospital.
- D. When assessing, treating, or transporting a Level III trauma patient, Paramedics may contact any SEMSC designated Trauma Center for medical direction. Paramedics may transport Level III patients to the closest Trauma Center.
- E. The use of air ambulances is considered separately from the trauma triage decision. Air ambulances may benefit patients injured in locations distant from Trauma Centers, and/or those in need of immediate procedures available to flight nurses, but outside the scope of practice of Paramedics. The use of air ambulances is not the default method of transport for major trauma patients. Aircraft should only be used when they offer a measurable advantage compared to ground transport. Use of air ambulances is covered in Policy 5800.
- F. Pregnant patients, greater than 24 weeks gestation, that do not meet Trauma Triage Criteria will be transported to a facility with OB capabilities.
- G. This policy does not apply to Multi-Casualty Incidents (MCIs).

IV. PARAMEDIC IMPRESSION

If the primary Paramedic believes that a patient not meeting criteria as a trauma patient has injuries that may exceed the capabilities of the usual receiving hospital, then the case should be discussed with SEMSC Designated Level II Trauma Base Hospital. The trauma base physician, Mobile Intensive Care Nurse (MICN), or designee, in consultation with the primary scene paramedic, may designate that patient as a trauma patient, and that patient will be transported to the nearest appropriate Trauma Center.

V. CRITERIA FOR TRUAMA PATIENT TRANSPORT TO THE CLOSEST RECEIVING FACILITY

The following trauma patients will be transported to the closest receiving facility:

- A. Trauma patients in cardiac arrest (consider field pronouncement);
- B. Trauma patients with an uncontrolled airway;
- C. Trauma patients with uncontrolled bleeding;
- D. Major trauma patients with rapid deterioration/impending arrest should be taken to the closest receiving facility if conditions (traffic, distance, weather) are unfavorable for rapid transport to a Solano County Designated Trauma Center.

VI. MAJOR TRAUMA PROCEDURES

- A. The primary Paramedic will determine whether the patient meets criteria as a trauma patient, and what level and type of trauma center care is appropriate for that patient (see Trauma Triage Algorithm attachment).
- B. The Primary Paramedic will determine the mode of transportation to the appropriate Trauma Center, in accordance with policy.
 - 1. If transportation is by ground, the transporting unit's dispatching agency will confirm that the closest destination Trauma Center is open.
 - 2. If transportation is by air ambulance, the air ambulance provider's dispatch will determine the closest appropriate destination Trauma Center, and confirm that it is open to receiving trauma patients.
- C. The transporting unit will contact the designated Trauma Center and advise them of their impending arrival, and provide a report on the patient's injuries and condition. Non-designated Out-of-county destination facilities are not authorized to give online medical instructions/orders.

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Solano County Health & Social Services Department

Mental Health Services
Public Health Services
Substance Abuse Services
Older & Disabled Adult Services



Eligibility Services
Employment Services
Children's Services
Administrative Services

Patrick O. Duterte, Director

Steven P. Whiteley, MD
EMS Agency Medical Director

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Ted Selby
EMS Agency Administrator

POLICY MEMORANDUM 6110

DATE: August 17, 2011

REVIEWED/APPROVED BY:

Steven P. Whiteley, MD
STEVEN P. WHITELEY, MD, EMS AGENCY MEDICAL DIRECTOR

Ted Selby
TED SELBY, EMS AGENCY ADMINISTRATOR

SUBJECT: REQUIRED MEDICAL RECORDS FOR PATIENT CARE

AUTHORITY: CALIFORNIA CODE OF REGULATIONS, DIVISION 9, ARTICLE 6, SECTIONS 100168, 100169(e); CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.220

I. PATIENT CARE REPORT (PCR)

Whenever an EMS (Emergency Medical Services) team arrives on scene of an incident where a patient is identified, and contact is made, a PCR will be generated.

EMS pre-hospital care providers are required to document all of their patient care on forms that are approved in writing by the EMS Medical Director and EMS Administrator. The Patient Care Report (PCR) Form is to be completed on all calls where a patient contact occurs. Advanced Life Support (ALS) Agencies will use an electronic PCR program that is California EMS Information System (CEMSIS) compliant, capable of producing a written document; a Solano County EMS Agency approved paper PCR version may be used when appropriate. A copy of the file will be provided to the receiving facility in printed or electronic form. In the event of computer failure, the agency will use the approved Solano County paper version of the PCR and enter all information into the electronic PCR database as soon as possible. If the computer down time is expected to exceed one week, the provider agency administrative representative will notify the EMS Agency in writing of the issue and include steps being taken to restore the electronic PCR and submission of data to the EMS Agency.

All EMS personnel are required to “document provision of all patient care and treatment provided utilizing an appropriate record.” This includes incidents where care is transferred to another EMS entity. Examples include but are not limited to:

- A. All calls where ALS and/or BLS care is provided.
- B. All (scheduled and non-scheduled) inter-facility transfers where EMS pre-hospital personnel are providing both continued assessment and treatment while a patient is being transferred between medical facilities.
- C. All cases where EMS pre-hospital personnel arrived on scene and initiated a patient assessment and/or treatment.
- D. All cases where the patient was found to be Dead on Arrival (DOA).
- E. All cases where the patient was identified as a Do Not Resuscitate (DNR) candidate for non-resuscitation.
- F. All cases where a physician assumes on-scene responsibility.
- G. Scheduled round trip inter-facility transfers to a diagnostic center (e.g., Cardiac Catheter Labs, MRI Scanners, CT Scan, specialty physician offices, etc.)

[*NOTE - If EMTs are required to make assessments (monitor vital signs) and/or maintain/administer therapeutic modalities (e.g., IV medications, apply direct pressure to control bleeding, monitor ECG dysrhythmias, etc), a PCR is required for each leg of the transfer.]

III. APPROVED MEDICAL CARE FORMS

The Solano County EMS Agency recognizes the following forms as acceptable for documenting EMS medical care provided by certified pre-hospital EMS providers:

- A. An approved CEMSIS-compliant Electronic PCR file.
- B. The Solano County Patient Care Report Form (and attached Continuation Form).
- C. Standardized Triage Tags for multi-casualty incidents; approved by the California Fire Chief's Association.
- D. The Standardized non-transport first responder report form.

IV. COMPLETION OF ELECTRONIC PCR FILE OR PAPER PCR

A call is not completed and a patient is not considered transferred to a physician's care until a PCR is completed and a copy provided to a receiving facility. An EMS unit is not available for pre-scheduled transfers until all records of medical care are provided to the physician caring for the patient. In the event an EMS unit needs to be dispatched to another emergency call, the EMS team may be released from their obligation to complete required medical records at that time. The treating EMS crew must provide the treating physician a way to contact them directly before leaving the receiving facility to allow the treating physician to ask additional

questions that only the EMS crew can answer. This may be accomplished via cellular phone or via radio.

In the event the EMS team responds to another emergency call without completing required medical records, the team must complete the PCR and fax as soon as possible, but in no case shall it be longer than 24 hours to submit the PCR to the appropriate medical facility. Failure to complete medical records is deemed abandonment.

V. DISTRIBUTION OF PAPER PCR FORM

- A. Provider Copy (1st Layer) – This is the original record maintained by the Ambulance Service Provider. For medical/legal purposes it is considered the master medical record and all other PCR layers are copies of the master. It is the responsibility of the Provider to maintain these patient records in accordance with medical liability statutes.
- B. Patient Medical Record Copy (2nd Layer) – This is the copy of the PCR that is retained by the receiving facility. In most cases this will be the receiving hospital or medical facility. In special cases this layer of the medical record may be left with public safety officials or other EMS/medical care providers (e.g., coroner's office for Dead-On-Arrivals (DOAs), nursing care facilities during transfers, medical staff at diagnostic facilities such as: Cardiac Cath Lab, CT Scan, Radiography, etc.).
- C. EMS Copy (3rd Layer) – This is the Local EMS Agency copy of the medical record utilized for statistical data processing. This copy is to be turned in to the Ambulance service provider's office and sent to the EMS office for processing by the 30th day of the following month.

VI. GENERAL GUIDELINES FOR MEDICAL RECORD DOCUMENTATION

Information needed to complete a PCR is dependent upon multiple sources and cannot always be precise and consistent (e.g., patient history, non-standardized clocks, multiple dispatch centers, inability to have a scribe on scene, multiple agencies responding to the scene incident, etc). PCRs are completed with EMTs making a good faith effort to appropriately and adequately complete accurate patient record keeping requirements.

If it is determined that any person alters or modifies a medical record, dispatch log, or any other EMS Quality Improvement documentation with fraudulent intent, or creates a false medical record either by including fraudulent information (e.g., vital signs reported but not taken) or omission/exclusion of essential information (e.g., missed IV or ETT attempts that are not recorded); this person is in violation of 471.5 of the California Penal Code and is subject to formal disciplinary action under 1798.200 of the California State Health & Safety Code.

If situations arise where there are multiple sources of documentation, the documentation provide by the individual(s) who actually performed the assessment, skill, treatment, etc. shall be the official legal document of record.

VIII. CONTINUATION FORM

Additional narrative information that exceeds the space provided in the paper PCR narrative section should be recorded on a Continuation Form provided by the Solano County EMS Office.

IX. ECG MONITORING STRIP

If ECG monitoring or a 12 lead EKG is a part of the patient's care and/or assessment, at a minimum one EKG strip or 12 lead EKG must be attached to the PCR either paper or electronic version of the PCR. The ECG strip or 12 lead EKG should be at least a six-second strip, with date and time noted on the strip. Multiple EKG strips or 12 lead EKGs should be sequential with the date and time noted as appropriate. Multiple EKGs or 12 lead EKGs should be attached to the PCR and submitted to the Receiving Facility and EMS Agency.

X. INSTRUCTIONS & CODE SHEET

Instructions and code sheets to complete the paper version of the Solano County PCR Form are included in Policy #6100.

XI BACK OF THE PCR (PROVIDER COPY)

- A. Intubation Report and verification (complete if applicable).
- B. Physician on Scene (complete if applicable):
Present the Physician with the *Solano County Note to Physician on Involvement with EMT and Paramedic* laminated card (Attachment 2) to explain the physician's options, and comply with Policy 6510, Physician on Scene.
- C. Solano County SIDS Report (Complete if applicable and fax to Solano County Coroner).

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PAPER PCR ELEMENTS and DESCRIPTION

#	CATEGORY	DESCRIPTION
1	EMS Agency - #48 (C0_01)	Can be automatically populated or could be reported electronically if not on PCR.
2	Provider Agency (E02_01) CEMSIS	Would be automatically populated by ePCR or hand entered. See codes below: 1. Benicia Fire Dept 4. Medic Ambulance Svc. 5. Vacaville Fire Dept. 6. Rio Vista Fire Dept. 7. Fairfield Fire Dept. 8. Cordelia Fire Dept 9. Dixon Fire Dept. 20. Other Ground Ambulance Agency permitted by EMS Agency 90. CALSTAR Air Ambulance 91. CHP EMS Aircraft (H-30) 92. REACH Air Ambulance 99. Other EMS Aircraft
3.	Unit # (E02_12) CEMSIS	Unique Solano County dispatch ID #.
4.	Incident # (E2_02) CEMSIS	The unique number provided by the agency responding to the call. May be pre-populated by ePCR.
5.	Date	The actual date of the call.
6.	Time of onset	When patient's symptoms started.
7.	PCR # (E01_01) CEMSIS	Unique number assigned by ePCR to each call or by provider.
8.	Primary method of payment (E07_01) CEMSIS	The primary method of payment or type of insurance associated with this EMS encounter. See list of codes below: -5 – Not available -10 – Unknown -15 – Not reported -20 – Not recorded -25 – Not applicable 720 – Private commercial insurance/HMO 725 – State Medicaid (Medi-CAL) 730 – Medicare 735 – Not billed 740 – Military insurance/other Government insurance 745 – Self pay or uninsured 750 – Workers' Compensation
9.	Incident Location (E08_12) CEMSIS	Location of incident must include city. This could be automatically pre-populated from ePCR or must be entered manually.

#	CATEGORY	DESCRIPTION
10.	Grid # or GPS (scene coordinates) (E08_10) CEMSIS	Enter the GPS coordinates of the incident location. Could be pre-populated in the ePCR.
11.	Incident Zip Code (E08_15) CEMSIS	Enter the incident location zip code.
12.	Patients' telephone number (Solano County specific)	Enter the patients' 10-digit telephone number.
13.	Patient's first name (Solano County specific)	Enter the patients' first name
14.	Patient's last name (E06_01) CEMSIS	Enter the patient's last or family name. Could be auto-populated by ePCR.
15.	Patient's address (Solano County specific)	Enter the patient's physical home address.
16.	Patient's city (Solano County specific)	Enter the patient's city where they live.
17.	Patient's state (Solano County specific)	Enter the state where the patient resides.
18.	Patient zip code (E06_08) CEMSIS	Enter the patient's zip code. This element could be extracted from the ePCR.
19.	Responsible party name (Solano County specific)	To be entered if the patient is a minor or under a conservatorship.
20.	Responsible party address (Solano County specific)	To be entered if the patient is a minor or under a conservatorship.
21.	Responsible party city (Solano County specific)	To be entered if the patient is a minor or under a conservatorship.
22.	Responsible party state. (Solano County specific)	To be entered if the patient is a minor or under a conservatorship.
23.	Responsible party zip code. (Solano County specific)	To be entered if the patient is a minor or under a conservatorship.
24.	Responsible party phone number. (Solano County specific)	To be entered if the patient is a minor or under a conservatorship.
25.	Age (E06_13) CEMSIS	Enter the patient's age.
26.	Age Units (E06_15) CEMSIS	Enter the units that best describe the patient's actual age (day, months, years).
27.	DOB (E06_16) CEMSIS	Enter the patient's date of birth (mm/dd/yyyy).
28.	Sex (E06_11) CEMSIS	Enter the patient's gender.

#	CATEGORY	DESCRIPTION
29.	Weight (E16_01) CEMSIS	Enter the patient's approximate weight in kilograms.
30.	PMD (Private Medical Doctor) (Solano County specific)	Enter the name of patient's regular MD (This information may be found on prescription bottles)
31.	Partial patient ID number (E06_10) CEMSIS	Enter the last five digits of the patient's social security number.
32.	Race (E06_12) CEMSIS	Enter the patient's race into this area. (C, N, A)
33.	Chief complaint	Enter the patient's chief complaint.
34.	Past medical history	Enter the patient's pertinent medical history in this area.
35.	Medications	Enter the patient's prescribed medications in this area.
36.	History of Sub/ History of ETOH (E12_19) CEMSIS	This box should be checked if there is evidence of drug or ETOH use at the scene or the patient indicates their use of controlled substances.
37.	Mechanism of Injury (E10_03) CEMSIS	This indicates if the type of injury is caused by blunt force, penetrating injury, or burn. See table below:
38.	Intent of Injury (E10_02) CEMSIS	If possible, the prehospital personnel can determine how the injury occurred.
39.	Cause of Injury (E10_01) CEMSIS	This provides information on the reported/suspected external cause of the injury.
40.	Physical exam (Solano County EMS Requirement)	Details the patient's physical exam finding. Physical exams should be done serially and recorded on the PCR.
41.	Medication (E16_03, E18_04, E18_05, E18_06, E18_07, E18_08) CEMSIS	These data elements provide information on medication administration to patients.
42.	Procedures (E19_03, E19_05, E19_06, E19_07) CEMSIS	These data elements provide information on procedures done by paramedics to patients.
43.	EKG (E14_03) CEMSIS	Document patient's initial cardiac monitor rhythm or any changes. A 6-second strip must be attached to the PCR.
44.	Patient's Vital Signs (E14_04, E14_05, E14_06, E14_07, E14_11, E14_12) CEMSIS	These are measured on patient. Serial vital signs should always be performed, time permitting.

#	CATEGORY	DESCRIPTION
45.	Pain scale (E14_23) CEMSIS	This documents the level of pain the patient is experiencing. Ten (10) is the maximum level of pain.
46.	Glasgow Coma Scale (GCS) (E14_19) CEMSIS	This documents the patient's initial level based on eye, verbal, and motor responses.
47.	Number of patients (E08_05) CEMSIS	This lists the number of patients at the scene of an incident.
48.	MCI (box) (E08_06) CEMSIS	Checking this box indicates the patient was a patient in a multi-casualty incident.
49.	Response Code & Transport Code (E02_20) CEMSIS	This indicates the code that responding unit used going to the scene and the code used transporting from the scene to destination.
50.	First Responder & aid performed before arrival. (E09_01, E09_02, E09_3) CEMSIS	This field indicates the care patients received by initial first responders and their agency.
51.	Primary clinical impression (E09_15) CEMSIS	This is the primary injury or illness the paramedic is treating the patient for.
52.	Secondary clinical impression (E09_16) CEMSIS	This is the secondary injury or illness the paramedic is treating the patient for.
53.	Receiving Facility (Solano County specific)	This field is to note the patient's destination.
54.	Reason for destination (E20_16) CEMSIS	This checkbox fields provide reasons why a specific destination was chosen.
55.	Narrative (Solano County specific)	This field allows the paramedic to document items/information not captured in any other field and provide a summary of the call.
56.	Times: (E05_03 - E05_07; E05_09 - E05_11; E05_13) CEMSIS	This is the area to document call times.
57.	Signature block and paramedic/EMT number (Solano County specific)	This field contains the treating paramedic/EMT signature and certification/accreditation number.

“Note To Physician” Laminated Card

	
COUNTY OF SOLANO	
NOTE TO PHYSICIAN ON INVOLVEMENT WITH EMT and PARAMEDIC	
A life support team (EMT or Paramedic) operates under standard policies and procedures developed by the local EMS agency and approved by their Medical Director under Authority of Division 2.5 of the California Health and Safety Code. The drugs they carry and procedures they can do are restricted by law and local policy. If you want to assist, this can only be done through one of the alternatives listed on the back of this card. These alternatives have been endorsed by CMA, State EMS Authority, CCLHO, and BMQA. Assistance rendered in the endorsed fashion, without compensation, is covered by the protection of the "Good Samaritan Code" (see Business and Professions Code, Sections 2144, 2395-2398 and Health and Safety Code, Section 1799.104). (over)	
(over)	

(FRONT)

ENDORSED ALTERNATIVES FOR PHYSICIAN INVOLVEMENT
After identifying yourself by name as a physician licensed in the State of California, and, if requested, showing proof of identity, you may choose to do one of the following: 1. Offer your assistance with another pair of eyes, hands, or suggestions, but let the life support team remain under base hospital control; or, 2. Request to talk to the base station physician and directly offer your medical advice and assistance; or, 3. Take total responsibility for the care given by the life support team and physically accompany the patient until patient arrives at a hospital and responsibility is assumed by the receiving physician. In addition, you must sign for all instructions given in accordance with local policy and procedure. (Whenever possible, remain in contact with the base station physician.)
Policy 6110; Attachment 2; (May, 2011) 

(BACK)

**Solano County Emergency Medical
Services Agency**

**Base Hospital Prehospital Report Form
Instruction Booklet**

Thomas L. Charron, MD, MPH
Solano County Health Officer/EMS Medical Director

717 Missouri St., Suite B
Fairfield CA 94533
(707) 421-6685

October 15, 1992
Ver 1.2

Attached is a copy of the revised Solano County Patient Care Report form. It was approved by the Solano County Health Officer/EMS Medical Director and is effective on or before September 30, 1992. The purpose of the form is to provide a standardized format to document all base hospital medical consultations with EMT-Ps and occasionally EMT-As requiring on-line medical direction for medical management of difficult cases. The record is the prime tool assisting the EMS Office in analysis of patient outcome of Advanced Life Support (ALS) patients that enter the EMS System. The record will be the basis for documenting and verifying a base hospital on-line medical consult as well as a tool to generate fees for services rendered to consumers receiving base hospital consultation services. The form will also be used as a tool to measure EMS System productivity and Q.I. (Quality Improvement) by allowing for efficient and more precise monitoring of MICNs and MDs who provide on-line medical control services. This form is to be used for all ALS and BLS emergency cases requiring base hospital consultation services. The form is to be used on the following cases:

- All ALS cases. This includes all ALS w/o base consult calls where paramedics initiate the majority or all ALS procedures without base consult.
- All emergency calls where a patient refuses emergency care and/or transport and the EMT-P/As in the field express concern that the patient needs treatment at an acute care medical facility.
- All DNAR/DOA cases requiring base hospital consult.
- All calls where a BLS ambulance requires on-line base hospital medical consult.
- All calls where the base hospital makes a medical determination that the patient does not need treatment and/or transport to an emergency department and that the patient is best served by treatment at an urgent care center, psychiatric facility, county clinic, private physician's office, or any other appropriate non-acute care setting.

(See Solano County Policy #6115, Required Base Hospital Medical Records for EMS Care). The form is not required for BLS calls where EMTs contact the hospital to inform the facility of their arrival and the need to prepare a bed for the incoming patient(s). The following directions are a step by step procedure for filling out the form. Please read the instructions carefully and refer to the attached examples illustrating the proper use of the forms. These instructions are also designed to provide you with new information about documenting the base hospital medical consultation. Any questions about the form or its use may be directed to the Solano County EMS Agency Office at (707) 421-6685. A sample copy of the report form is on the following page.

Solano County Emergency Medical Services Agency Base Hospital Prehospital Report Form

Date (1)	Time (2)	Tape No. (3)	Base Hosp. (4)	MICN (5)	Auth # (6)	Base Hospital MD. (7)	
Chief Complaint (9)				Age (10)	Sex (11)	Weight (12)	Paramedic & Company (13)

Pertinent Medical History and/or Special Scene Circumstances:

(14)

Medications:

Allergies:

Private MD.

Initial Vital Signs	Level of Con.	Lung Sounds	Pupils	Degree of Distress																												
Pulse: _____ Resp: _____ BP: _____ ECG: _____	☐ Oriented x 3 ☐ Responds to voice ☐ Responds to pain ☐ Unresponsive ☐ Unconscious PTA ☐ Disoriented	<table style="width: 100%;"> <tr> <th>Left</th> <th>Right</th> </tr> <tr> <td>☐ Clear</td> <td>☐</td> </tr> <tr> <td>☐ Wheezes</td> <td>☐</td> </tr> <tr> <td>☐ Rales</td> <td>☐</td> </tr> <tr> <td>☐ Rhonchi</td> <td>☐</td> </tr> <tr> <td>☐ Diminished</td> <td>☐</td> </tr> <tr> <td>☐ Absent</td> <td>☐</td> </tr> </table>	Left	Right	☐ Clear	☐	☐ Wheezes	☐	☐ Rales	☐	☐ Rhonchi	☐	☐ Diminished	☐	☐ Absent	☐	<table style="width: 100%;"> <tr> <th>Left</th> <th>Right</th> </tr> <tr> <td>☐ PERL</td> <td>☐</td> </tr> <tr> <td>☐ Sluggish</td> <td>☐</td> </tr> <tr> <td>☐ Constricted</td> <td>☐</td> </tr> <tr> <td>☐ Dilated</td> <td>☐</td> </tr> <tr> <td>☐ Unresponsive</td> <td>☐</td> </tr> <tr> <td colspan="2">Pupil Size _____</td> </tr> </table>	Left	Right	☐ PERL	☐	☐ Sluggish	☐	☐ Constricted	☐	☐ Dilated	☐	☐ Unresponsive	☐	Pupil Size _____		☐ Severe ☐ Moderate ☐ Mild ☐ None ☐ N/A
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☐ Constricted	☐																															
☐ Dilated	☐																															
☐ Unresponsive	☐																															
Pupil Size _____																																

(16)

Skin	Head/Neck	Chest	Abdomen	Pelvis	Extremities	Back
☐ Normal ☐ Pale/Ashen ☐ Flushed ☐ Cyanotic ☐ Diaphoretic ☐ Dry ☐ Hot ☐ Cool	☐ Negative	☐ Negative	☐ Negative	☐ Negative	☐ Negative	☐ Negative

Treatment/Orders ☐ ALS Treatment was initiated before Base Contact ☐ C-Spine Precautions Oxygen: _____

Time Treatment/Orders Patient's Response BP Pulse Resp ECG

(19)

(20)

☐ Check This Box If Case Required Activation of a Trauma Team (Activation Time _____)

Destination (21)	ETA (22)	Code in & Transport By ALS/BLS (23)	Signature-MICN or MD performing base consult (24) (25)
Type of EMS Consult & ALS Protocols Ordered (26)	EMS PCR# (27)	EMT-P Clinical Impression (28)	
Arrival Time (30)	Emergency Department Diagnosis (31)	Discharge Time (32)	ED Disposition (33)
		QA Follow-up Codes (34)	

Complete this section if receiving hospital was not the base.

Person Contacted: _____

Arrival Time: (30) Discharge Time: (32) ED Disposition: (33)

EMG Dept Diagnosis: (31)

Patient Name: (35)

Medical Record #: (36)

Insurance Payor Code: (37) Billing No. (38)

Base QA Follow-up Codes: (34)

Patient Identification Addressograph

Solano County Base Hospital Prehospital Report Form
Data Element Instruction List

1. **Date** - is indicated by month, day and year. eg. 03/23/88
2. **Time** - is the time the EMT-P or EMT-A first contacted the base hospital requesting a base consult. (Use a 24 hour clock to record time intervals. Seconds are not necessary)
3. **Tape Number** - as indicated. A unique tape identifier is used at or by each base hospital to record each base consultation and assist in identification of the case if it is selected for retrospective medical review. Each base hospital has a unique system to number tapes, please contact your base liaison nurse to assist you in how to use their individual numbering system.
4. **BASE HOSPITAL:** Enter the ID of the base hospital as appropriate.

1.	NorthBay Medical Center (NBMC)
2.	Sutter-Solano Medical Center (SSMC)
41.	Sutter-Davis Medical Center (SDMC)
32.	Methodist Hosp. Sacramento (MHS)
5. **MICN** - Enter the name of the MICN who directly managed the case.
6. **MICN Authorization #** - Enter the unique Solano County MICN Authorization # of the individual who managed the case.
7. **Base Hospital MD** - Enter the name of the MD who is medically responsible for the case. It is not necessary to note whether physician directly talked to the field team or orders were relayed through an MICN.
8. **Authorization #** - Enter a unique identification # of the MD who managed the case. The number should be a unique identification number (or billing ID#) assigned by the Base Hospital or EMS Agency which is assigned to the physician after the individual has been trained and oriented to the EMS System and the EMT-P medical protocols. A number is needed to allow for proper billing of on-line medical control services rendered.
9. **Chief Complaint** - refers to the **patient's** impression of the major symptoms responsible for the ambulance call relayed by the EMT-P. The EMT-P should provide the patient's chief complaint as part of the initial communication with the base hospital. The chief complaint is not the EMT-Ps **clinical impression** which should also be communicated distinctly to the base hospital. There are likely to be many instances when the patient's chief complaint and the EMT-Ps clinical impression are consistent. (eg. Patient's chief complaint is respiratory distress and the EMT-Ps clinical impression is the same.) There are other instances when a patient's chief complaint is only a part of and/or different from the EMT-Ps clinical impression. (eg. a trauma patient with multiple injuries, chest pain, ALOC cases, 51-50 cases etc.). The EMT-P should report the patient's chief complaint and also provide a clinical impression including all pertinent findings obtained from the initial assessment. Examples of chief complaints include: shortness of breath, right leg pain, headache, chest pain on inspiration, fainting etc. The following would not be considered typical chief complaints: fractured femur, CVA, and acute MI. These findings are more often thought to be part of the EMT-Ps clinical impression. If the patient is unconscious, indicate as the chief complaint for this type of emergency.

10. **Age** - refers to patient's age in years, use "mos" when months are indicated,(eg. 6 mos)
11. **Sex** - refers to male or female, using M or F.
12. **Weight** - refers to patient's estimated weight as reported by the EMT, use Kg when indicating kilograms.
13. **Paramedic and Company** - Enter the EMT-P authorization number and/or name which should be provided in the consult and the code of the ambulance company. The EMT-P should provide his/her initial EMT-P authorization number at the time of initial contact with the base hospital. Each ambulance company has a code number assigned it by the Solano EMS Office. Enter either the actual company name or its EMS code number (See below) with the EMT-Ps auth.#.

Solano County EMS Service Providers Codes

- | | |
|-----|-------------------------------------|
| 1. | Benicia Fire Department |
| 3. | Foothill Ambulance Service |
| 4. | MEDIC Ambulance Service |
| 5. | Solano Ambulance Service |
| 6. | Vacaville Fire Department |
| 7. | Mare Island Fire Department |
| 8. | Winters Fire Department |
| 50. | Other (Non-listed ground transport) |
| 90. | CAL-STAR |
| 91. | UCD Life Flight |
| 92. | CHP Helicopter |
| 99. | Other EMS Air Transport Service |

14. **Pertinent Medical History and/or Special Scene Circumstances** -refers to the EMT-P's summation of the medical problem, and any other information that pertains to the problem relevant to the immediate management of the case. This section may include such information as: what brought on the symptoms? as well as descriptions of events surrounding the accident, secondary complaints, past medical history which relates to the present illness, major related medical problems, recent use of medications, observations by bystanders, and details of the accident scene such as "bent steering wheel." Physical finding which the EMT-P obtains from observing or examining the patient, belong within the physical findings section, but may be included in this section as a function of transcribing the EMT-Ps initial report. **Medications** refers to any prescription medication taken regularly by patient if reported by the EMT-P. If none, state so. If unknown, or not reported, state so. **Allergies** refers to any substance to which the patient has a known allergy. If none, state so. **Private Medical Doctor** - enter name as appropriate, if provided by the EMT-P.
15. **Initial Vital Signs** - Enter as appropriate BP, Pulse, Resp. Rate, ECG, and other significant physical assessments reported by the EMT-P requesting on-line medical direction from the base hospital. **Level of Consciousness, Lung Sounds, Pupil Response, Skin Color, Skin, Head/Neck, Chest, Abdomen, Pelvis, Extremities, and Back** - Check boxes as appropriate, enter any special clinical finding(s) as appropriate that are provided by the EMT-P requesting the base consult.

1. **Degree of Distress** - Check as appropriate. Enter the MICN/MD's impression of the patient's acuity that is determined after hearing and evaluating the EMT-P's initial presentation to the base hospital.
17. **ALS Treatment Was Initiated Before Base Contact:** - Check this box if the EMT-P elected to provide any ALS care prior to initiation of his/her base consultation.
18. **C-Spine Precautions** - Check this box if the EMT-P reports that cervical spinal immobilization occurred as part of the initial prehospital care. **Oxygen Section** - Enter data as appropriate if oxygen was administered.
19. **Treatment/Orders and Patient's Response Section** - use this space to record your medical orders and patient's response as reported by the EMT-P.
20. **Activation of a Trauma Team** - Check the box and enter the activation time as appropriate.
21. **Destination (Receiving Facility)** - Enter the name of the facility or the appropriate code listed below:

Solano County EMS Receiving Facility Codes

0. N/A non-transport case

In County Receiving Facilities

- 1. North Bay Medical Center
- 2. Sutter Solano Medical Center
- 3. Kaiser Permanente Med Center
- 4. VacaValley Hospital
- 5. David Grant Medical Center
- 9. Other Non-Hospital Receiving Facility

Out of County Receiving Facility

- 20. Napa County Hospital Facility
- 21. Queen of the Valley Medical Center
- 30. Sacramento County Hospital Facility
- 31. UCD Medical Center
- 40. Yolo County Hospital Facility
- 41. Sutter Davis Medical Center
- 50. Contra Costa County Hospital
- 51. John Muir Medical Center
- 90. Other Bay Area Medical Facility
- 91. Santa Clara Valley Medical Center
- 92. San Francisco General Hospital
- 93. UC San Francisco
- 94. Oakland Children's Hospital
- 95. R.K. Daves Medical Center
- 99. Other

22. **ETA (estimated Time of Arrival)** - as provided by the EMT-P.
23. **CODE-IN** - should be the transport mode reported by the EMT-P.

- 2. In Code 2 (stable transport without lights and siren).
- 3. In Code 3 (unstable transport requiring lights and siren). EMS Aircraft transports are Code 3 cases.
- N/A Not applicable (for non-transport cases).

24. **Transport by ALS/BLS** - Identify the transporting ambulance service if it is different from the EMS Service Provider identified in #7 listed above. Skip this element if the same EMS service provider requesting the initial consult is also directly responsible for transporting the patient(s).
25. **Signature of MICN or MD Performing Base consult** - The MICN/MD signature should be the actual signature of the individual who was directly responsible for the base communications with the EMT-P. The ID number is the individual's Solano County EMS authorization number needs to be included to identify the MICN/MD in the computer system who actually renders the on-line medical consultation services with prehospital personnel.
26. **Type of EMS Consult** - Enter a code number as appropriate.

Type of EMS Consult MIS Code Numbers

1. ALS call (w/ground transport)
2. ALS call (w/air transport)
3. BLS call (notification of arrival)
4. DOA on scene and/or terminate resuscitation
5. DNAR call requiring base consult
6. AMA call requiring base consult
7. MCI/Medical disaster situation
8. Consult confirming EMS transport is not necessary or medically appropriate.
9. Other unusual circumstances requiring consult.

27. **ALS Protocols Ordered** - (See code list below - enter as appropriate)

Behavioral Emergencies

- B1 Psychiatric Crisis
B2 Child Abuse/Sexual Assault

Cardiac Emergencies

- C1 Shock (Non-traumatic)
C2 Shock (Non-trauma & Resp.Dist.)
C3 Cardiac Arrest (V-fib/V-Tach)
C4 Cardiac Arrest (PIVR,EMD,ASY)
C5 Vent Tachy with Pulses
C6 Bradycardia with Pulses
C8 Paroxysmal Supravent Tach
C9 Other Cardiac Dysrhythmia
C10 Chest Pain (Cardiac)
C11 Chest Pain (Non-Cardiac)
C12 Hypertensive Emergencies

Environmental Emergencies

- E1 Heat Illness/Hyperthermia
E2 Hypothermia/Frostbite
E3 Envenomation
E4 Burns
E5 Near Drowning/Crowning

HAZ-MAT Emergencies

- H1 General Procedures
H2 Irritant Gases (Acids,Ammonia,Chlorine)
H3 Hydrofluoric Acid
H4 Smoke Inhalation/Carbon Monoxide
H5 Arsine/Phosphine Gas
H6 Cyanide
H7 Hydrogen Sulfide, Sulfides and Mercaptan)

Other HAZ-MAT Protocols

- H8 Petroleum Distillates and Halogenated Hydrocarbon Solvents
- H9 Pesticides (Carbamates and Organophosphates)

Medical/Neurologic Emergencies

- M1 Abdominal Pain
- M4 Allergic Reaction
- M5 Anaphylaxis
- M6 Poisons/Overdose
- N1 Coma\ALOC
- N2 Diabetic Complications
- N3 Seizures
- N4 CVA/Stroke
- N5 Syncope

Obstetric Emergencies

- O1 Vaginal Hemorrhage with Shock
- O2 Vaginal Hem W/O Shock
- O3 Imminent Delivery, N Present
- O4 Breech Present/Prolapsed Cord
- O5 Pre-Eclampsia\Eclampsia

Pediatric Emergencies

- P1 Ped Cardiac Arrest (V-fib/Tach)
- P2 Ped Cardiac Arrest (EMD/AS)
- P3 Pediatric Hypotension
- P4 Neonatal Resuscitation
- P5 Pediatric Bradycardia

Respiratory Emergencies

- R1 Airway Obstruction
- R2 Croup/Epiglottitis
- R3 Acute Respiratory Distress
- R4 Respiratory Arrest
- R5 COPD
- R6 Asthma\Bronchospasm
- R7 Acute Pulmonary Edema
- R8 Spontaneous Pneumothorax
- R9 Toxic Gas Inhalation

Traumatic Emergencies

- T1 Traumatic Shock
- T2 Traumatic Cardiac Arrest
- T3 Head and Neck Trauma
- T4 Chest Trauma
- T5 Abdominal Trauma
- T6 Extremity Trauma

Special Procedures

- S1 Pleural Decompression
- S2 High Dose Epinephrine
- S3 Rectal Diazepam (Valium)
- S4 Needle Cricothyrotomy

- 28. **EMS PCR#** - Enter the unique patient care report form number or Triage Tag Number as appropriate.
- 29. **EMT-P Clinical Impression** - Enter as appropriate.
- 30. **Arrival Time** - Enter the time the patient arrived at his/her receiving facility. Enter N/A for non-transport cases.

31. **Emergency Department Diagnosis** - Enter as appropriate. This data element is one that will have to be entered after the patient is discharged from the Emergency Department.
32. **Discharge Time** - Enter the time the patient was discharged from the receiving facility. This data element is one that will have to be entered after the patient is discharged from the Emergency Department. Enter N/A for non-transport cases.
33. **Emergency Department Disposition** - Enter the ED Disposition code when the patient discharged from the receiving facility. This data element is one that will have to be entered after the patient is discharged from the Emergency Department. Enter N/A for non-transport cases.

Emergency Department Disposition Codes

1. Home (Disch from ED)
2. DOA and Died in ED
3. AMA withdrew from ED
4. N/A direct admit bypass ED

Patient is admitted to the Hospital

10. General Med/Surg Admit
11. ICU
12. CCU
13. Pediatric\Neonatal Unit
14. Operating Room\Surgical Unit
15. Acute Psychiatric Unit
16. Alcohol Detox Unit
17. Drug Detox Unit
18. Obstetric Unit\Birth Center
19. Other special in hospital unit

Transferred from the ED (w/receiving hospital code)

30. Gen Med/Sur Unit
31. ICU
32. CCU
33. Burn Unit
34. PICU/Peds/Neonatal Unit
35. Trauma Unit/Service
36. Acute Spinal Cord Unit
37. Acute Psychiatric Unit
38. Alcohol Detox Unit
39. Drug Detox Unit
40. Obstetric/Peri-Natal Unit
41. Neuro-Surgical ICU
42. Other (Pt., MD. request)
43. Re-implantation Center
99. Other

EMS QA Follow-up Codes - Enter single or multiple codes as appropriate.

Base Liaison Q.A. Follow-up Codes

1. Neurosurgical EMG Case
2. On-Line Request Audit Request
3. Major Trauma Patient
4. Cardiac Arrest Pt. surviving initial field resuscitation
5. Mass Casualty Incident Patient
6. HAZ-MAT incident with patient injury
7. EMT-Defibrillation case successfully admitted
8. Other (Special Base Hospital Study)
9. Other (Special Base Hospital Study)

Additional Data for Cases Transported to Non-Base Hospitals

35. **Patient Name** - Enter as appropriate for information submitted by the receiving hospital.
36. **Medical Record #** - Enter the unique case number used by the receiving hospital to identify the patient for later retrospective review, if indicated.
37. **Payor Codes** - Enter as appropriate from information submitted by the receiving hospital.

0. Unknown
1. Kaiser
2. Other Com. Insurance (PPO/HMO.etc.)
3. Medicare
4. State Medicaid, MEDI-CAL, (include pending MEDI-CAL)
5. Champus* (Other Military Insurance)
6. Worker Compensation
7. Public Assistance
8. Self pay\Uninsured
9. Other
99. N\A

38. **Billing Number** - Enter as appropriate from information submitted by the receiving hospital.

Patient Identification Addressograph - Items 35 - 38 are available on the addressograph stamp for cases taken to the base hospital.



Health & Social Services Department

Donald R. Rowe, Director

Public Health - Mental Health - Public Guardian
Adult and Child Services - Eligibility & Employment Services
Substance Abuse Services

EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #6115

EFFECTIVE DATE: 11/01/92

APPROVED BY:

Thomas L. Charron
THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

Steven J. Mahinski
STEVEN J. MAHINSKI, M.D., F.A.C.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

SUBJECT: REQUIRED BASE HOSPITAL MEDICAL RECORDS FOR EMS CARE

AUTHORITY: CALIFORNIA CODE OF REGULATIONS, DIVISION 9, ARTICLE 6, SECTION 100167(b)4, 5, 8, 13; 100168(b)2; CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.220

PURPOSE: To establish the requirements and written instructions for Base Hospital documentation of EMS calls.

I. BASE HOSPITAL LOG/REPORT OF EMS BASE HOSPITAL CONSULTATION

Base Hospital MICNs and/or MDs directing ALS field care are required to document their medical consults on medical records/forms that are approved in writing by the EMS Medical Director and/or the Solano County Health Officer. At the present time, either the Base Hospital Log and/or the Base Hospital Prehospital Report Forms are approved forms that are to be completed on all calls where an EMS service provider requests a Base Hospital consultation. Calls that should have a Base Hospital Consultation record include:

- A. All emergency calls where ALS care is provided.
- B. All (scheduled and non-scheduled) inter-facility transfers where EMS pre-hospital personnel encounter a problem and/or patient's clinical status deteriorates requiring Base Hospital consultation and/or direction.

- C. All emergency calls where ALS care is provided.
- D. All (scheduled and non-scheduled) inter-facility transfers where EMS pre-hospital personnel encounter a problem and/or patient's clinical status deteriorates requiring Base Hospital consultation and/or direction.
- E. All incidents where there are multiple patients (MCI Incident) and/or a medical disaster. (In this type of an event, standard Base logs and/or Base Report Forms should be substituted with standard medical ICS report/records recommended by the Solano County OES (Office of Emergency Services), BAMMA (Bay Area Medical Mutual Aid Plan), or the Hospital Council.
- F. All cases where an EMS aircraft transports a patient to an appropriate receiving facility as directed by the Base Hospital.
- G. All cases where the ambulance arrived on scene, a patient was assessed but there was a refusal of care and/or transport even when the EMT-P or EMT-A on scene recommends transport to a medical facility.
- H. All cases where a Base consult was necessary to determine the discontinuation of CPR.
- I. All cases where the patient was identified as a DNR candidate and where the DNR form is inappropriate for the field situation encountered.
- J. Any and all cases where EMS pre-hospital personnel require medical consult and/or assistance to appropriately medically manage a field event.
- K. All EMS cases where an EMT-P elected to institute ALS care, even if the Base Hospital did not have any time to provide medical direction or concur with actions the EMT-P took prior to Base consult.
- L. All EMS cases where an EMT-P/EMT contacts the Base Hospital for a determination whether or not ambulance transport is medically appropriate.

II. EXCEPTIONS TO COMPLETING BASE HOSPITAL LOG/REPORT:

- A. BLS situations when an ambulance transporting a patient is contacting a Base Hospital/receiving facility to notify the medical staff that their arrival is imminent.
- B. Medical consult and/or orders are not indicated or requested by the ambulance unit contacting the Base (BLS Calls).
- C. Notification of patient arrival is the prime reason for using medical communications (non-ALS calls).
- D. All BLS emergency calls where current BLS treatment protocols appropriate to meet the patient's medical needs.
- E. All scheduled transfers where no assessment and/or treatment was provided by the transporting teams (e.g., transfers to/from convalescent facilities, Cardiac Cath Labs, MRI scanners, CT-Scan, specialty physician offices, etc.).

- F. Interfacility transfers where sending hospitals provide nursing and medical staff to monitor and manage the patient during transfer (e.g., NICU transports, EMS aircraft transfers, etc).

III. APPROVED BASE HOSPITAL LOG/REPORT MEDICAL CARE FORMS

Currently the Solano County EMS system recognizes the following forms as acceptable for documenting EMS medical care provided by certified pre-hospital EMS providers:

- A. The Solano County Base Hospital Communication Logs (Currently being phased out).
- B. The Solano County Base Hospital Prehospital Report Form or other Base Hospital Prehospital Report Form approved by EMS.

IV. COMPLETION OF THE BASE HOSPITAL LOG/REPORT MEDICAL CARE FORMS

The Base Hospital medical records should be completed concurrently while medical consult is being provided to the patient. Because of the nature of the prehospital environment and the severity of illness/injury of the patient, it may be completed immediately after the patient has been received at a medical facility and no later than prior to ending the Base MICN's shift. The Base Hospital medical records require final outcome data about the patient after being released from the Emergency Department. This information may be added later with clerical assistance and/or entered by the Base MICN responsible for the patient prior to ending the Base MICN's shift. Each Base Hospital shall establish a procedure to insure patient outcome data is included on all cases requiring medical consult.

V. DISTRIBUTION OF THE BASE HOSPITAL PREHOSPITAL REPORT FORM

- A. Medical Records Copy (1st Layer) – This is the original record and is maintained at the Base Hospital and/or may be forwarded to the receiving hospital. For medical/legal purposes it is considered the master medical record of the incident. It is the responsibility of the Base Hospital to maintain these patient records in accordance with medical liability statutes for medical records that are in effect for hospitals. Receiving facilities may elect not to include Base Hospital medical records within their patient medical records. The Patient Medical Record (PCR) copy left at the receiving facility is the definitive medical record of the prehospital care provided that patient. Any distribution of the Base Hospital Prehospital Report Form should be based on a mutually agreed upon schedule between the Base and the receiving facility. Records of non-transports and/or cases in which the receiving facility elects not to receive may be destroyed by the Base 30 days after the call.
- B. Base Copy (2nd Layer) – This is the copy of the Base Hospital Prehospital Report Form to be retained by the Base Hospital, medically responsible for on-line medical control of the case, for retrospective and medical control review.

VI. GENERAL GUIDELINES FOR BASE HOSPITAL MEDICAL RECORD DOCUMENTATION

- A. There should never be a conflict between patient care and charting. Patient care always takes precedence, but the paperwork must still be done before the MICN and/or the Base Physician shifts end.
- B. Charts must be legible.
- C. Charting should be completed as soon as possible, no later than close of shift, except for Emergency Department diagnosis and patient outcome data (within the grey area) which should be completed within 24 hours.
- D. Document facts, not conclusions or opinions.
- E. Correct charting errors with a line through the error and a notation with your initials, date and time.
- F. Use military time. It is expected that EMTs and MICNs cannot use precise time intervals for all assessment and treatments. If you have to approximate times when charting, do so with an "@" symbol before the time (e.g., @15:45).

VIII. CONTINUATION FORM

Additional narrative information that exceeds the space in the PCR narrative section should be recorded on a Continuation Form provided by the Solano County EMS Office.

IX. INSTRUCTIONS & CODE SHEETS

Instructions and code sheets to complete the Solano County Patient Care Report Form are enclosed in Policy #6100.

Solano County Health & Social Services Department

Mental Health Services
Public Health Services
Substance Abuse Services
Older & Disabled Adult Services



Eligibility Services
Employment Services
Children's Services
Administrative Services

Patrick O. Duterte, Director

EMERGENCY MEDICAL SERVICES AGENCY

275 Beck Ave., MS 5-240

Fairfield, CA 94533

(707) 784-8155 FAX (707) 421-6682

www.solanocounty.com

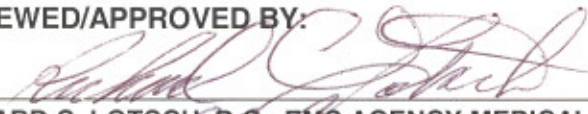
POLICY MEMORANDUM 6125

Michael A. Frenn
EMS Agency Administrator

Richard C. Lotsch, D. O.
EMS Agency Medical Director

DATE: July 25, 2006

REVIEWED/APPROVED BY:


RICHARD C. LOTSCH, D.O., EMS AGENCY MEDICAL DIRECTOR


MICHAEL A. FRENN, EMS AGENCY ADMINISTRATOR

SUBJECT: REFUSAL OF MEDICAL ASSISTANCE

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, Chapter 5, § 1797.220 & 1798.

INTENT: It is the expectation and intent in Solano County EMS system that a paramedic or EMT completes an appropriate assessment in order to determine if an individual is a patient. That assessment will also establish the level of medical care which needs to be rendered and properly documented. The policy also outlines the quality assurance requirements.

PURPOSE/POLICY:

- I. Purpose:
 - A. To determine when a person is identified as a patient in the EMS system.
 - B. To establish a standard process for the termination of the EMS Provider/patient relationship.
 - C. To identify the minimally required assessment and the necessary documentation required on the patient care record (PCR) when a patient terminates the EMS Provider/patient relationship.
 - D. To identify what resources are available for the EMS Provider to utilize when a patient is encountered who, in the EMS Provider's judgment, should not terminate the relationship.
 - E. To identify whom other than the patient can make the determination that a patient does not need to be treated and/or transported to the hospital.

- F. To establish quality assurance standards and reporting requirements for EMS Service Providers.

II. Definitions

- A. Patient: A Patient means any person who meets any of the following:
1. Has a physical complaint: The individual has a complaint of recent or new onset such as pain, shortness of breath, or weakness;
 2. Has obvious injury: The individual has signs of injury such as cuts or abrasions following a traumatic event;
 3. If the individual specifically called for or requests medical evaluation and/or care;
 4. Is possibly under the influence of drugs or alcohol or exhibits any impairment in sensorium that a reasonable medical provider would consider to be abnormal.
 - a. EMS personnel must use extreme caution when persons have evidence of being under the influence of drugs or alcohol. These substances can mask serious injuries and illnesses.
 5. Has been involved in an incident, or has experienced a mechanism, with potential for serious injury such as:
 - a. A motor vehicle crash with intrusion into passenger space, broken windshield, bent steering wheel, or damaged dashboard;
 - b. Ejection from a vehicle;
 - c. Rollover incident involving unrestrained persons;
 - d. A motorcycle or other wheeled vehicle crash with damage to helmet, speed greater than 20 mph or separation of the rider from the vehicle;
 - e. A pedestrian (or rider of a wheeled vehicle) struck by a vehicle traveling at any speed;
 - f. Has an altered mental status (recent or current);
 - g. A person who is unconscious or has a recent history of loss of consciousness;
 - h. A person who is not fully oriented to person, place or time;
- B. A Non-Patient refers to:
1. Any person that paramedics or public safety personnel encounter who does not demonstrate any known or suspected illness or injury and/or who has not been subjected to a significant mechanism of injury as defined above.
 2. If there is no patient as defined in section II.A, this policy does not apply and neither a Patient Care Record (PCR) nor Refusal of Medical Assistance (RMA) Form is required. Non-patient encounters must be documented on the Solano County Non-Patient Encounter Form (Atch 3).
- C. Competency: The ability to understand and to demonstrate an understanding of the nature and consequences of refusing medical care.

- D. An “adult patient” is:
 - 1. At least 18 years of age; or
 - 2. A minor (under age 18) that is lawfully married, divorced, or had an annulment; or
 - 3. A minor on active duty with the armed forces; or
 - 4. A legally emancipated minor with documentation provided by a court of law; or
 - 5. A self-sufficient minor at least 15 years of age, living apart from parents and managing own financial affairs regardless of source or lack of source of income and regardless of parental consent or lack of consent to the separation.
- E. Designated Medical Decision Maker:
 - 1. An individual other than the patient who has the legal responsibility for making the patient’s medical decisions.
 - 2. This responsibility does not automatically fall to the spouse or the relatives unless there is a legal designation.
 - 3. The parent or legal guardian for an individual under the age of 18 who does not meet the definition above for an adult.
 - 4. An individual who has a signed consent from the parent for medical decisions of a minor such as a teacher, school counselor or nurse or a camp counselor.
 - 5. An individual who is designated as the attorney in fact for decisions regarding the health care for the patient
- F. Emergent Medical Care: Treatment to prevent loss of life, treatment of serious bodily injury, or treatment to alleviate severe pain.

III. Application

- A. This policy applies to any patient who refuses medical assistance or transportation. The paramedic shall document the patient’s refusal on the PCR and obtain a Refusal of Medical Assistance form that is signed by the patient or the person legally designated to make decisions regarding the health care for the patient.
- B. Only one Refusal of Medical Assistance Form is necessary for each patient.
- C. The patient should initiate any discussion regarding refusal of care and/or transport.
- D. All questions asked by the patient or the patient’s legal medical decision maker related to cost or appropriateness of care should be answered to the best of the paramedic’s ability and documented on the PCR.

IV. PATIENTS ALLOWED REFUSAL OF MEDICAL ASSISTANCE

- A. In order to be competent to refuse medical assistance, a patient must be able to understand and to demonstrate an understanding of the nature and consequences of refusing medical care.

- B. The following patients are allowed to refuse medical assistance:
1. A competent adult
 2. A minor patient who meets the definition of adult as defined in section II.D. above
 3. A competent parent or guardian on behalf of a minor.
 4. The patient's designated medical decision maker with written documentation of conservatorship of the person or the patient's durable power of attorney for health care.
 5. Some adult patients may make a decision to refuse care that EMS personnel feel is irrational, but the patient is felt to be otherwise competent. If the EMT-P is concerned that the patient is not making decisions as a reasonable person would in the same situation, the EMT-P should attempt to prevail upon the patient to consent to treatment. Other resources which may be useful in convincing the patient to agree to care include:
 - a. Enlist family members to assist in convincing the patient to consent to care
 - b. Contact a base hospital physician to speak with the patient directly
 - c. Law enforcement personnel may be able to convince the patient or place them on a 5150 hold when appropriate

V. PATIENTS NOT ALLOWED REFUSAL OF MEDICAL ASSISTANCE/TRANSPORT

- A. Minor patients except as defined in section II.D. above.
- B. In the following situations, when a patient is refusing care, intervention of law enforcement personnel is indicated to determine whether the provisions of the policy regarding a 5150 hold should apply.
1. Any patient who presents with altered level of consciousness
 2. Any patient who has attempted suicide, or verbalizes suicidal intent
 3. Any patient whose judgment is impaired due to drugs/ETOH
- C. Patients who are on a 5150 hold are not allowed to/cannot refuse transportation or emergency medical care, but may refuse treatment for non-emergency conditions.
- D. Law enforcement personnel can also assist in making appropriate disposition of minors who are refusing transportation, but do not meet the requirements for a 5150 hold.

VI. Process:

- A. Determine that the individual meets the definition of a patient and is competent to refuse care and sign the Refusal of Medical Assistance Form
- B. Clearly offer both treatment and transportation to the hospital.
- C. Attempt to perform a physical assessment that includes a complete set of vital signs
- D. Obtain and document a history of the event that led to the 911 call. When possible,

include prior medical history including medications.

- E. Explain the benefits of medical treatment.
- F. Explain the risks of refusal of medical treatment and/or transportation.
- G. Determine that the patient has an understanding of the risks and benefits of treatment and transport.
- H. Advise the patient or the patient's legally designated medical decision maker to seek medical attention for any and all medical complaints.
- I. Advise the patient or the patient's legally designated medical decision maker to call 911 if the condition continues or worsens.
- J. Determine whom if anyone will be present with the patient.
- K. PCR Documentation:
 - 1. A Solano County PCR must be completed on all patients who refuse medical assistance.
 - 2. Documentation will include at a minimum, the items listed above in section VI. B. through VI. J.
- L. Refusal of Medical Assistance (RMA) Form (Atch 2):
 - 1. Prepare and explain the RMA Form to the patient or the patient's designated medical decision maker.
 - 2. Have the patient or the patient's legal representative sign the RMA Form.
 - 3. The signature of the patient or the patient's legally designated medical decision maker should be witnessed if possible.
 - 4. The paramedic should attempt to have a preferred witness sign the RMA Form. Preferred witnesses include:
 - a. A member of the patient's family
 - b. An individual selected by the patient as a witness
 - c. A member from a public service agency who has not been directly responsible for the patient's assessment and treatment
 - d. An EMS provider that has not filled out the PCR
 - 5. Should the patient refuse to sign the RMA Form, two individuals who witnessed the interaction should sign the RMA Form and there should be documentation of the circumstances surrounding the refusal of service on the PCR.
 - 6. If the patient leaves the scene or refuses to listen to the explanations involved in obtaining the RMA Form, the circumstances will be carefully documented on the PCR and no RMA Form will be completed.
- M. Non-Patient Encounter Form (Atch 3)
 - 1. Non-patient encounters must be documented on the Solano County Non-Patient Encounter Form.

VII. Special Situations

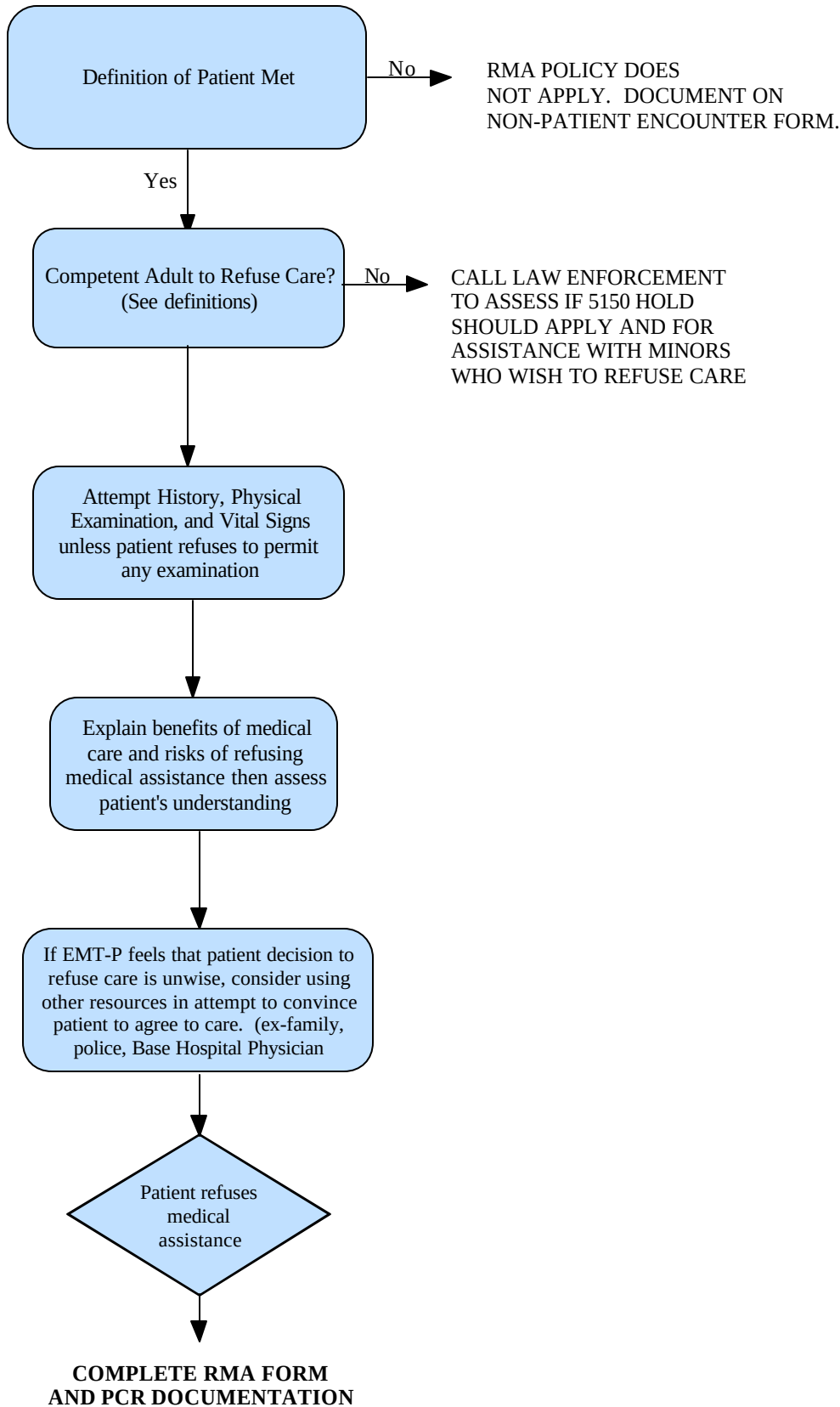
- A. All special circumstances will be documented on the PCR.
- B. If a patient with an emergent medical condition is refusing care and appears competent, a base hospital physician consultation must be obtained with the hope that the physician will speak with the patient and be able to convince the patient to consent to care.
- C. In order for a non-English speaking patient to refuse care an explanation must be provided and instructions given in the patients own language by the paramedic or through an interpreter.
- D. If the patient or the patient's legally designated medical decision maker is unable to read the form, the Refusal of Medical Assistance Form should be read to the patient in the appropriate language.
- E. If a parent or the patients legally designated medical decision maker refuses care and has no immediate plan for care of a minor child, elder or dependent adult who is in distress, paramedics shall request law enforcement assistance.
- F. If the paramedic suspects child abuse, the abuse of an elder or dependent adult or domestic violence the paramedic shall request law enforcement assistance. In addition, the paramedic is required by law to file a report with the appropriate social service agency (see Solano EMS Policy 6190).

VIII. Quality Assurance Requirements for EMS Service Providers

- A. EMS Service Provider shall maintain a medical quality assurance (Q.A.) monitoring system that shall review all cases in which there was a pre-hospital refusal of medical assistance.
- B. EMS Service Provider shall audit the medical records of each case in which EMTs or paramedics encountered a pre-hospital refusal of medical assistance using audit criteria in Attachment A within 15 days of the incident date
- C. EMS Service Provider shall provide counseling to its employees if the audit determines medical records are inconsistent with expectations.
- D. EMS Service Provider shall submit the audit summary to their Provider Medical Director if it determines that any of the medical audit criteria were present and a Base Hospital consult did not occur.
- E. Reporting Requirements: At the end of each month, each EMS Service Provider shall submit actual audits and/or a computerized data file of the months' RMA audits in conjunction with the PCRs that are submitted to the Solano EMS Agency office.

#

ATCH 1: REFUSAL OF MEDICAL ASSISTANCE PROCEDURE



**SOLANO COUNTY EMS
REFUSAL OF MEDICAL ASSISTANCE FORM**

EMS Service:	Date:	Time:
Patient Name:	Age:	Phone #
Incident Location:	Incident #	
Situation of EMS Call:	NON PATIENT: ☐ (see Non-Patient Encounter Form)	

PATIENT ASSESSMENT:

Any current medical complaint: ☐ Yes ☐ No (If yes – describe: _____)
Suspected injury or illness based on patient history, physical examination or mechanism of injury: ☐ Yes ☐ No

*** Check marks in shaded areas should prompt law enforcement assessment for protective custody or 5150 Hold before patient release. ***

Competency to Refuse Medical Assistance:

18 years of age or older: ☐ Yes ☐ No Patient Oriented to: <table style="width: 100%;"><tr><td>Person:</td><td>☐ Yes</td><td>☐ No</td></tr><tr><td>Place:</td><td>☐ Yes</td><td>☐ No</td></tr><tr><td>Time:</td><td>☐ Yes</td><td>☐ No</td></tr><tr><td>Event:</td><td>☐ Yes</td><td>☐ No</td></tr></table>	Person:	☐ Yes	☐ No	Place:	☐ Yes	☐ No	Time:	☐ Yes	☐ No	Event:	☐ Yes	☐ No	Any evidence of: <table style="width: 100%;"><tr><td>Suicidal?</td><td>☐ Yes</td><td>☐ No</td></tr><tr><td>Head Injury?</td><td>☐ Yes</td><td>☐ No</td></tr><tr><td>Intoxication?</td><td>☐ Yes</td><td>☐ No</td></tr><tr><td>Any altered mental status?</td><td>☐ Yes</td><td>☐ No</td></tr><tr><td>Mentally impaired in any way?</td><td>☐ Yes</td><td>☐ No</td></tr></table>	Suicidal?	☐ Yes	☐ No	Head Injury?	☐ Yes	☐ No	Intoxication?	☐ Yes	☐ No	Any altered mental status?	☐ Yes	☐ No	Mentally impaired in any way?	☐ Yes	☐ No
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Mentally impaired in any way?	☐ Yes	☐ No																										

Risks explained to patient: _____
Patient understands clinical situation and risks ☐ Yes ☐ No
Patient verbalizes understanding of risks: ☐ Yes ☐ No
Patient's plan to seek further medical evaluation: _____
Who will be with the patient after EMS departure? _____

LAW ENFORCEMENT ASSESSMENT FOR 5150 (if applicable):

AGENCY: _____ Officer: _____ Badge # _____

BASE STATION CONTACT:

Physician: _____ BASE STATION: _____ TIME: _____
Base Physician spoke to patient: ☐ Yes ☐ No
Base Physician Orders: _____

PATIENT OUTCOME:

_____ Patient refuses transportation to a hospital against medical advice;
_____ Patient accepts transportation to hospital by EMS but refuses any or all treatment offered.
Treatment refused: _____
_____ Other: (Explain): _____

This form is being provided to me because I have refused assessment, treatment and/or transportation by EMS personnel for myself or on behalf of this patient. I understand that EMS personnel are not physicians and are not qualified or authorized to make a diagnosis and that their care is not a substitute for that of a physician. I recognize that there may be a serious injury or illness which could get worse without medical attention even though I (or the patient) may feel fine at the present time. I understand that I may change my mind and call 911 if treatment or assistance is needed later. I also understand that treatment is available at an emergency department 24 hours a day. I acknowledge that this advice has been explained to me by the EMS Personnel and that I have read this form completely and understand its terms.

Signature (patient or other) Date EMS Provider Signature

If other than patient, print name and relationship to patient

PROVIDER COPY

**SOLANO COUNTY EMS
REFUSAL OF MEDICAL ASSISTANCE FORM**

EMS Service:	Date:	Time:
Patient Name:	Age:	Phone #
Incident Location:	Incident #	
Situation of EMS Call:	NON PATIENT: ☐ (see Non-Patient Encounter Form)	

PATIENT ASSESSMENT:

Any current medical complaint: ☐ Yes ☐ No (If yes – describe: _____)
Suspected injury or illness based on patient history, physical examination or mechanism of injury: ☐ Yes ☐ No

*** Check marks in shaded areas should prompt law enforcement assessment for protective custody or 5150 Hold before patient release. ***

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Patient verbalizes understanding of risks: ☐ Yes ☐ No

Patient's plan to seek further medical evaluation: _____

Who will be with the patient after EMS departure? _____

LAW ENFORCEMENT ASSESSMENT FOR 5150 (if applicable):

AGENCY: _____ Officer: _____ Badge # _____

BASE STATION CONTACT:

Physician: _____ BASE STATION: _____ TIME: _____

Base Physician spoke to patient: ☐ Yes ☐ No

Base Physician Orders: _____

PATIENT OUTCOME:

_____ Patient refuses transportation to a hospital against medical advice;

_____ Patient accepts transportation to hospital by EMS but refuses any or all treatment offered.

Treatment refused: _____

_____ Other: (Explain): _____

This form is being provided to me because I have refused assessment, treatment and/or transportation by EMS personnel for myself or on behalf of this patient. I understand that EMS personnel are not physicians and are not qualified or authorized to make a diagnosis and that their care is not a substitute for that of a physician. I recognize that there may be a serious injury or illness which could get worse without medical attention even though I (or the patient) may feel fine at the present time. I understand that I may change my mind and call 911 if treatment or assistance is needed later. I also understand that treatment is available at an emergency department 24 hours a day. I acknowledge that this advice has been explained to me by the EMS Personnel and that I have read this form completely and understand its terms.

Signature (patient or other)

Date

EMS Provider Signature

If other than patient, print name and relationship to patient

PATIENT COPY

**SOLANO COUNTY EMS
REFUSAL OF MEDICAL ASSISTANCE FORM**

EMS Service:	Date:	Time:
Patient Name:	Age:	Phone #
Incident Location:		Incident #
Situation of EMS Call:		NON PATIENT: ☐ (see Non-Patient Encounter Form)

PATIENT ASSESSMENT:

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Who will be with the patient after EMS departure? _____

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AGENCY: _____ Officer: _____ Badge # _____

BASE STATION CONTACT:

Physician: _____ BASE STATION: _____ TIME: _____

Base Physician spoke to patient: ☐ Yes ☐ No

Base Physician Orders: _____

PATIENT OUTCOME:

_____ Patient refuses transportation to a hospital against medical advice;

_____ Patient accepts transportation to hospital by EMS but refuses any or all treatment offered.

Treatment refused: _____

_____ Other: (Explain): _____

This form is being provided to me because I have refused assessment, treatment and/or transportation by EMS personnel for me or on behalf of this patient. I understand that EMS personnel are not physicians and are not qualified or authorized to make a diagnosis and that their care is not a substitute for that of a physician. I recognize that there may be a serious injury or illness which could get worse without medical attention even though I (or the patient) may feel fine at the present time. I understand that I may change my mind and call 911 if treatment or assistance is needed later. I also understand that treatment is available at an emergency department 24 hours a day. I acknowledge that this advice has been explained to me by the EMS Personnel and that I have read this form completely and understand its terms.

Signature (patient or other)

Date

EMS Provider Signature

If other than patient, print name and relationship to patient

NON-PATIENT ENCOUNTER FORM

ENCOUNTER CHECKLIST:

Yes No

- | | | |
|--------------------------|--------------------------|---|
| ☐ | ☐ | Has a physical complaint: The individual has a complaint of recent or new onset such as pain, shortness of breath, or weakness |
| ☐ | ☐ | Has obvious injury: The individual has signs of injury such as cuts or abrasions following a traumatic event |
| ☐ | ☐ | If the individual specifically called for or requests medical evaluation and/or care |
| ☐ | ☐ | Has been involved in an incident, or has experienced a mechanism, with potential for serious injury such as: <ul style="list-style-type: none">a. A motor vehicle crash with intrusion into passenger space, broken windshield, bent steering wheel, or damaged dashboardb. Ejection from a vehiclec. Rollover incident involving unrestrained personsd. A motorcycle or other wheeled vehicle crash with damage to helmet, speed greater than 20 mph or separation of the rider from the vehiclee. A pedestrian (or rider of a wheeled vehicle) struck by a vehicle traveling at any speed |
| ☐ | ☐ | Has an altered mental status (recent or current) |
| ☐ | ☐ | A person who is unconscious or has a history of fainting or seizure |
| ☐ | ☐ | A person who is not fully oriented to person, place or time |
| ☐ | ☐ | Is possibly under the influence of drugs or alcohol or exhibits any impairment in sensorium. |

- If a **“YES”** is marked, a PCR **must** be completed.
- If ALL are marked **“NO”**, then complete TOP section of Provider Copy **only**.

NOTE: The above checklist must be completed on all potential Non-Patient encounters. If any of the above items is marked **“YES”**, this person is a **“PATIENT”**.

Solano County Health & Social Services Department

Mental Health Services
Public Health Services
Substance Abuse Services
Older & Disabled Adult Services



Eligibility Services
Employment Services
Children's Services
Administrative Services

Gerald Huber, Director

Pranav Shetty, MD, MS, MPH
EMS Agency Medical Director

EMERGENCY MEDICAL SERVICES AGENCY
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Benjamin Gammon, EMT-P
Interim EMS Agency Administrator

POLICY MEMORANDUM 6140

REVISED DATE: March 31, 2022

REVIEWED/APPROVED BY:

Pranav Shetty, MD, MS, MPH, EMS Agency Medical Director

Benjamin Gammon, EMT-P, Interim EMS Agency Administrator

SUBJECT: RESPONSIBILITY FOR TRANSFER OF PATIENT CARE

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, §1797.204, §1797.220

I. PURPOSE/POLICY:

To establish guidelines for the transfer of patient care responsibility from a first responder Paramedic to a transporting Paramedic or EMT.

II. INTENT:

The general expectation and intent in the Solano County EMS system is that patient care will be transferred from the first responder Paramedic to a transporting Paramedic, or EMT, when it is medically prudent and safe for the patient. There are some instances in Solano County wherein a Transfer of Care (TOC) may not take place from an initial responding paramedic to a transport paramedic or EMT. Field care is a team effort; however, a team leader must be identified. It is the responsibility and obligation of each team member to participate in the patient's care and provide information, support, and suggestions to the team leader to enhance and provide safe patient care.

The aforementioned TOC represents the transfer of patient management authority. This policy assumes that a first responder Paramedic has arrived in advance of the transporting Paramedic and has been able to perform an adequate assessment and obtain history in order to determine a probable course of action for the patient, and that the two Paramedics are from different provider agencies. If insufficient time has elapsed to permit an adequate history and assessment, transfer of patient care should be initiated as quickly as possible.

If the transport Paramedic arrives prior to or simultaneous with the first responder Paramedic, patient management authority shall rest with the transport Paramedic.

Nothing in this policy is intended to alter the current procedure of transfer of patients that occurs in the use of aeromedical resources.

III. TRANSFER OF CARE (TOC)

A pre-hospital provider (EMT or Paramedic) who initiates treatment or establishes care or assessment of a patient in the field shall attend to the patient until such time as patient care is transferred, or the patient is delivered to an acute care facility, with the following exceptions:

- A. The patient refuses medical assistance in accordance with Policy #6125.
- B. The patient requires only Basic Life Support (BLS) level treatment, and the patient is being transported by an EMT staffed ambulance (applies only when a Paramedic initiates the assessment or care of a patient in the field).

IV. PROCEDURE FOR TRANSFER OF CARE (TOC):

- A. TOC should not occur in the following instances:
 - 1. The patient has been intubated, defibrillated, and/or cardioverted; or
 - 2. The patient is determined to be in critical condition; or
 - 3. Either the transferring or receiving Paramedic is uncomfortable in transferring patient care.
- B. The first responder Paramedic and the transport Paramedic may pursue exceptions to the above with concurrence from the Base Hospital if, in their judgment, patient care will not be compromised.
- C. Transfer of patient care shall occur from one Paramedic to another and initiated at an appropriate point in the management of the patient as determined by the first responder Paramedic. The time of transfer of care shall be documented on all PCRs.

- D. A complete verbal report of the patient's history, assessment, and treatment rendered shall be given to the transporting Paramedic by the first responder Paramedic.
- E. The following documentation shall be required:
 - 1. A PCR from the non-transporting Paramedic on every patient shall be completed and copied to the EMS Agency and the receiving hospital within one-hour of the incident. Every effort should be made to satisfy this requirement as soon after the incident as possible.
 - 2. A PCR from the transporting paramedic according to existing policy.

V. TERMINATION OF ADVANCED LIFE SUPPORT (ALS) CARE

At no time shall ALS procedures be terminated on a patient for the sole purposes of utilizing Basic Life Support (BLS) transfer and transport without the expressed concurrence by the Base Hospital Physician on duty. This shall only occur under extraordinarily unusual circumstances when such ALS care is unavailable. This shall be considered an Unusual Occurrence in the quality assurance system, followed by the appropriate Q. A. Documentation with submission of a Field Advisory Report (FAR) in accordance with Policy 2305.

VI. ALS TRANSFER IN EMT STAFFED AMBULANCE

An EMT staffed ambulance may transport a patient under the care of other health care professionals where those professionals are using ALS methods or procedures. The responsibility and documentation for the patient care shall remain with the health care professional delivering such advanced life support care. In this event a FAR should be submitted to the EMS Agency within 48-hours.

VII. PARAMEDIC TRANSFER ASSISTED BY OTHER HEALTHCARE PROFESSIONALS

A Paramedic, during interfacility transfer of a patient in company with a physician or Registered Nurse, may at the direction of the physician or Registered Nurse, assist in the delivery of assessments, procedures, or administration of medication which are within the scope of practice of a Paramedic, including but not limited to: establishing IV access; intubation; sterile suction; ventilation using oxygen delivery systems; childbirth. Such intervention, assessments, etc., shall be documented on the prehospital Patient Care Report and be signed by the physician or Registered Nurse in addition to the Paramedic. The Base Hospital shall remain available for information and advice.

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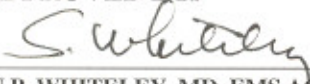
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Steven P. Whiteley, M.D.
EMS Agency Medical Director

POLICY MEMORANDUM 6143

DATE: 9/1/2005

REVIEWED/APPROVED BY:


STEVEN P. WHITELEY, MD, EMS AGENCY MEDICAL DIRECTOR


MICHAEL A. FRENN, EMS AGENCY ADMINISTRATOR

SUBJECT: PATIENT RESTRAINT POLICY

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, SECTION 1798.220.

PURPOSE:

Provide guidance in the use of restraints in the field or during transport for patients who are violent, may become violent, or may potentially harm themselves or others. Restraints are used to ensure the protection and safety of both rescuers and patients

I. DEFINITIONS:

- A. Restraint: refers to the process of the physical hindrance of a violent or combative patient from harming himself/herself or others when verbal means of control have failed
- B. Control Devices or Restraints: The type of limb-holding device used to prevent the patient from harming himself/herself. The limb-holding device used on the patient shall be made of leather or cloth. Handcuffs will not be used by field care personnel.

II. TYPES OF RESTRAINTS

- A. Authorized types of restraints for EMS Personnel:
 - 1. Commercially manufactured soft restraints.
 - 2. Commercially manufactured leather/Velcro-type
 - 3. Sheets
 - 4. Seat belt type restraints.
- B. Unauthorized types of restraints for EMS Personnel:
 - 1. Choke holds.
 - 2. Plastic ties
 - 3. Handcuffs
 - 4. "Hobbles" or "Hog-tied" restraints.

III. PROCEDURE

- A. Scene safety is the first priority for the rescuers. If law enforcement presence is needed, then the rescuers should wait until law enforcement arrives before patient contact is initiated, if possible.
- B. Always attempt to verbally de-escalate a patient's aggressive or assaultive behavior with a calm and reassuring approach to patient care.
- C. There are a number of medical conditions that can cause a patient to act in a dangerous or assaultive manner. Obtain a detailed medical history, including drug and alcohol use, from any family members, bystanders or law enforcement personnel in the vicinity.
- D. Restraints should only be used as a last resort and should be placed on the patient in a non-antagonistic manner.
- E. Once the decision is made to use restraints on a patient, use the minimum number of EMS personnel required, the minimum amount/type of restraint necessary and the minimum amount of force.
- F. The restraints shall not be placed in such a way to preclude evaluation of the patient's medical status (e.g., airway, breathing, circulation), necessary patient care activities, or in any manner that will jeopardize the patient medically.
- G. Unless there is an urgent medical indication for the immediate removal of the restraint, they should not be removed until sufficient personnel are available to maintain control of the patient.
- H. If the patient has been handcuffed, placed in plastic ties or "hogtied", the patient should be accompanied by a law enforcement officer in the back of the ambulance, possible. If it is not possible then law enforcement will directly follow the ambulance in their patrol car.
- I. Notify the Base Hospital of the use restraints to control a violent patient.

IV. CONTINUOUS QUALITY IMPROVEMENT

- A. The following information will be annotated on the PCR:
 - 1. Time of Base Physician contact.
 - 2. Type of restraint used on the patient.
 - 3. Specific reason(s) for the application of restraints.
 - 4. Annotate the "Airway, Breathing, Circulation Assessment" before and after the patient is placed into restraints, at a minimum. This assessment will be completed frequently on the restrained patient.
 - 5. Monitor restrained extremities for circulation, motor and sensory functions every fifteen (15) minutes and annotate on the PCR.
- B. Any situation that falls outside "Normal Operations" will have an "Unusual Occurrence" form completed and filed, per Solano County Policy 2300.